Collectively Big, Locally Small

Formalizing Small Data in Healthcare and Learning Privately Across Siloed Hospitals

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Health systems produce data at an enormous rate, yet most clinical learning problems are small. A rare disease registry holds a few hundred patients, an intensive care unit sees a handful of the outcome it wants to predict, a regional hospital owns a fraction of the cases needed to validate a model, and the data that would close the gap sit in other institutions that cannot legally or practically share them. "Small data" is invoked constantly in the literature but is rarely defined in a way that can be measured, compared, or reasoned about. This survey and position paper does four things. First, it gives a task-relative formal definition of small data through an effective sample size and a smallness index that combine learning-theoretic sample complexity with the clinical sample-size criteria used in prediction modeling. Second, it introduces a new two-axis taxonomy. The first axis classifies ten forms of scarcity in healthcare by the term of the smallness index they degrade, grouped into ontic, epistemic, temporal-contextual and custodial scarcity. The second axis classifies every remedy by the provenance of the information it injects (pooling, proxy, prior, unlabeled data, procurement, re-expression), and we prove that re-expression, which includes augmentation and synthetic data, cannot add information about the target that was not already present in the data or in the generator. Third, we code a corpus of 290 works on both axes, check it against a systematic search of 5,204 database records reported to PRISMA 2020, and use it to review the solution families across imaging, electronic health records, omics and tabular data, to summarize the quantitative evidence, and to locate where it is thin. Only 18 of the 290 works are multi-institution studies, all of them on pooling, and temporal obsolescence and measurement poverty are barely addressed by any channel, in the corpus and in the wider literature alike. Fourth, for sparse networks of hospitals that operate as data silos, we derive a privacy-preserving hierarchical borrowing scheme that runs by gossip over the existing collaboration graph without a central server. A companion paper turns it into a method that, without tuning, was more accurate than federated and personalized learning tuned on other networks in a pre-registered test on unseen cancer networks. We show that, in expectation over sites, it adds nearly the same number of "virtual patients", $\sigma^2/(\tau^2 + M)$, to every site, so that the smallest hospitals gain the most, that differential privacy enters only through the population-level estimate, and that the gossip phase incurs no additional privacy cost, and we summarize eighteen simulations and a first test on real data from four hospitals, reported in full in a companion paper, against one-shot and iterative federated baselines and under adversarial conditions. We close with a research agenda.

CCS Concepts: • **Computing methodologies** → **Machine learning**; • **Applied computing** → **Health informatics**; • **Security and privacy** → *Privacy protections.*

Additional Key Words and Phrases: small data, healthcare, effective sample size, taxonomy, federated learning, decentralized learning, differential privacy, hierarchical Bayes, data silos, rare diseases, foundation models, synthetic data

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1 INTRODUCTION

The standard story of machine learning in medicine is a story about scale. Large labeled collections of retinal photographs, skin lesions and chest radiographs produced models that matched specialists on narrow tasks [Rajpurkar et al. 2022; Topol 2019], and the broader field learned that performance tends to grow predictably with data [Halevy et al. 2009; Hestness et al. 2017; Kaplan et al. 2020; Sun et al. 2017]. The story is accurate for the problems it describes. It is not representative of the problems most clinicians and health systems actually face.

Consider what a typical clinical question looks like from the inside. Rare diseases are individually rare but collectively common: the Orphanet database lists more than six thousand of them, affecting an estimated 3.5% to 5.9% of the population [Nguengang Wakap et al. 2020], and most have cohorts of tens or hundreds of patients worldwide [Banerjee et al. 2023; Schaefer et al. 2020]. Prediction models in common conditions are routinely built on a few dozen outcome events, which is why the clinical literature has spent three decades arguing over events per variable [Peduzzi et al. 1996; Riley et al. 2019b; van Smeden et al. 2019]. During the first months of COVID-19, hundreds of models were developed from small, local, convenience samples, and systematic reviews found nearly all of them to be at high risk of bias and none fit for clinical use [Roberts et al. 2021; Wynants et al. 2020]. Even large systems turn small at the point of deployment: a proprietary sepsis model used in hundreds of hospitals achieved an area under the curve of 0.63 on external validation and missed two thirds of sepsis cases [Wong et al. 2021], and convolutional networks for pneumonia learned to recognize the hospital that produced an image rather than the disease [Zech et al. 2018].

These failures share a structure. Health data are abundant in aggregate and scarce where they are needed: for the specific outcome, in the specific subgroup, at the specific site, in the current period, with labels. They are also fragmented. Every hospital holds a slice of the population under its own legal, ethical and technical constraints [Cohen and Mello 2018; European Parliament and Council of the European Union 2016; Price and Cohen 2019], and the European Health Data Space regulation [European Parliament and Council of the European Union 2025] is a recent recognition that this fragmentation is itself a policy problem. The consequence is a paradox that motivates this paper: *healthcare is collectively big and locally small.*

The term "small data" appears in social science [Kitchin and Lauriault 2015], in personal health [Estrin 2014; Hekler et al. 2019], in statistics [Faraway and Augustin 2018], and in hundreds of applied papers. It is almost always used informally, as a synonym for "fewer samples than we would like". Three consequences follow. Methods are compared across problems whose difficulty is not comparable. Remedies that cannot work in a given setting are applied anyway, as when synthetic data is expected to compensate for a cohort that does not contain the relevant information. And the specific case of many small, isolated institutions, which is the default organizational form of healthcare, is treated either as a federated learning problem with a central server that often does not exist, or not at all.

Contributions. This paper is both a survey and a position paper. Its contributions are as follows.

(1) **A formal definition.** We define small data relative to a task, through an *effective sample size* n^{eff} that accounts for clustering, missing labels, label noise, distribution shift and outcome rarity, and a *smallness index* $\sigma = n^{\star}/n^{\text{eff}}$ that compares it with the sample complexity $n^{\star}$ of the task (Section 2). The definition assembles known ingredients (Kish's design effect, the sample

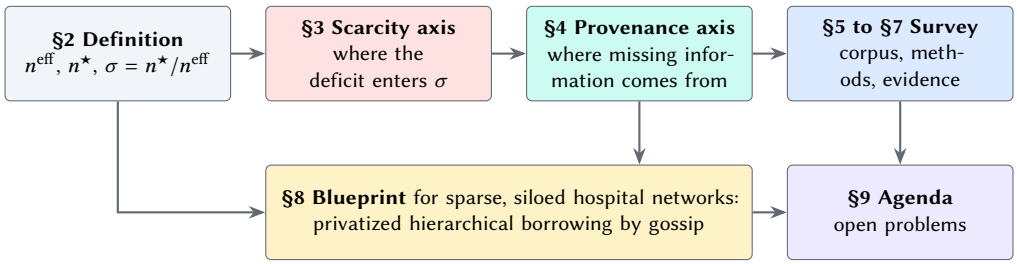


Fig. 1. Structure of the paper. The definition of Section 2 produces the two axes of the taxonomy, which serve as the coding scheme of the survey and shape the design of the blueprint.

complexity of learning with label noise, and the sample-size criteria used for clinical prediction models) into one task-relative quantity. It is a first-order heuristic for diagnosis and comparison, not a new estimator.

(2) **A new taxonomy.** We propose the Scarcity-Provenance taxonomy (Sections 3 and 4). Its first axis classifies ten forms of scarcity in healthcare by the term of σ they degrade. Its second axis classifies remedies by where the missing information comes from. We prove a "no free lunch" result for re-expression (Proposition 4.1) that explains formally why augmentation and synthetic data cannot by themselves solve a small-data problem.

(3) **A coded survey.** We classify a curated corpus of 290 works on both axes (Section 5; the full coding is in the online supplement), review the solutions channel by channel (Section 6), and compare where the literature has actually worked with where the taxonomy says it could, which exposes specific empty cells (Section 7).

(4) **A blueprint for siloed hospital networks.** For sparse networks of hospitals without a trusted central server, we derive a privacy-preserving hierarchical borrowing scheme with closed-form risk, explicit privacy cost and convergence on sparse graphs (Section 8), argue that it is a principled default for this setting, and summarize its evaluation in eighteen simulations and a pre-registered confirmation on real cancer networks (Section 8.7). The method for realistic model heads, its full privacy analysis and the evaluation are developed in a companion paper [Simões et al. 2026]. The ingredients are standard; what is new is their combination for serverless hospital networks, and its evaluation against federated and personalized methods tuned as a consortium could tune them.

(5) **A research agenda** with concrete open problems (Section 9).

Scope and method. We cover supervised and semi-supervised prediction from clinical data: imaging, structured electronic health records (EHR), clinical text, omics and tabular registries. We do not cover causal effect estimation from small trials in depth, although several of our results apply to it. How the corpus was assembled and coded, and how a systematic database search reported to PRISMA 2020 complements it, is described in Section 5. Existing surveys cover individual families, such as federated learning in healthcare [Nguyen et al. 2022; Rieke et al. 2020; Xu et al. 2021], few-shot learning [Wang et al. 2020], meta-learning [Hospedales et al. 2022], imperfect medical datasets [Cheplygina et al. 2019; Tajbakhsh et al. 2020], informed machine learning [von Rueden et al. 2023] and synthetic data [Chen et al. 2021a; Jordon et al. 2022]. Among the surveys we found, none defines small data formally, none organizes remedies by information provenance, and none treats the sparse, serverless hospital network as a first-class setting.

Table 1. Main notation.

Symbol	Meaning
$\mathcal{T} = (P, \mathcal{P}, \mathcal{H}, \ell, \varepsilon, \delta)$	learning task: target distribution, family of distributions the learner must handle, hypothesis class, loss, tolerance, confidence
n, n^{eff}	nominal number of records; effective sample size (Definition 2.2)
$n^{\star}_{\mathcal{T}}$	sample complexity of $\mathcal{T}$ (Definition 2.1)
$\sigma_{\mathcal{T}}(\mathcal{D})$	smallness index $n^{\star}_{\mathcal{T}}/n^{\text{eff}}(\mathcal{D})$ (Definition 2.3)
DEFF, ρ_L, η, ρ_S, π	design effect, labeled fraction, label noise rate, shift efficiency, outcome prevalence
K, $\mathcal{G} = (V, E)$, W	number of hospitals, collaboration graph, gossip matrix
θ_k, $\hat{\theta}_k$, $v_k = \sigma^2/n_k$	parameter of site k, its local estimate, and the variance of that estimate
μ, τ^2, M	population mean of site parameters, between-site heterogeneity, mean squared error of the estimate of μ
(ϵ, δ)	differential privacy parameters (written ϵ to distinguish them from the tolerance ε)

How the parts connect. The paper is built as a single argument, shown in Figure 1. We first make smallness measurable (Section 2). The terms of that measure tell us *why* a dataset is small, which gives the scarcity axis of the taxonomy (Section 3). Knowing why points to *where* the missing information could come from, which gives the provenance axis (Section 4). The two axes then serve as the coding scheme for a corpus of 290 works (Section 5), which organizes the survey of existing methods (Section 6) and the reading of their evidence (Section 7). The gaps that this reading exposes, together with the definition of siloed smallness from Section 2, determine the design of the blueprint (Section 8), and what remains unsolved becomes the agenda (Section 9).

2 WHAT IS SMALL DATA? A FORMAL ACCOUNT

The paradox of the introduction can only be studied once it can be measured. This section turns "small" from an adjective into a quantity, and that quantity is the thread that runs through the rest of the paper.

2.1 Existing notions

Informal notions of small data fall into three groups. The first is *absolute*: a dataset is small if it has few records, often with arbitrary cut-offs such as a few hundred or a few thousand. This ignores the task. One thousand patients is a lot for estimating a mean blood pressure and very little for a 50-predictor model of a 2% outcome. The second is *relational*, as in Kitchin and Lauriault's contrast between small data, collected for a specific question with controlled sampling, and big data, which is exhaustive, fine-grained and collected as a by-product [Kitchin and Lauriault 2015]. Estrin's "n of 1" [Estrin 2014] and the small data paradigm of Hekler et al. [2019] belong here, stressing individual-level inference rather than population scale. Faraway and Augustin [2018] show that a small designed sample can beat a large biased one, which is a statement about information, not about size. The third group is *criterion-based* and comes from clinical prediction research: a dataset is adequate if it contains at least ten events per candidate predictor [Peduzzi et al. 1996], or more defensibly if it meets the criteria of Riley et al. [2019b], which bound overfitting, optimism in apparent fit and imprecision of the intercept [Riley et al. 2020; van Smeden et al. 2019].

Only the third group is quantitative, and it is tied to regression models. Learning theory provides the general quantity, sample complexity, but it ignores the specific ways in which clinical data lose information. We combine the two.

2.2 Setup and notation

A learning *task* is a tuple $\mathcal{T} = (P, \mathcal{P}, \mathcal{H}, \ell, \varepsilon, \delta)$, where P is a distribution over $\mathcal{X} \times \mathcal{Y}$ describing the target population at the point of use, $\mathcal{P} \ni P$ is the family of distributions that are consistent with what is known about the task before seeing data, $\mathcal{H}$ a hypothesis class, ℓ a loss bounded in $[0, 1]$, and (ε, δ) the required excess risk and confidence. The risk of h under Q is $R_Q(h) = \mathbb{E}_{(x,y)\sim Q}\, \ell(h(x), y)$, and $R = R_P$. The family $\mathcal{P}$ is needed because P itself is unknown: a learner that knew P could output the best hypothesis without any data, so sample complexity is only meaningful as a guarantee that holds for every distribution the learner cannot rule out. Notation used throughout is collected in Table 1.

Definition 2.1 (Sample complexity of a task). The sample complexity $n^\star_{\mathcal{T}}$ is the smallest n for which there is a learner A such that, for every $Q \in \mathcal{P}$ and $\mathcal{D} \sim Q^n$ independent and identically distributed (i.i.d.) and fully and correctly labeled,

$$\Pr_{\mathcal{D}\sim Q^n}\left[R_Q(A(\mathcal{D})) - \inf_{h\in\mathcal{H}} R_Q(h) \le \varepsilon\right] \;\ge\; 1 - \delta.$$

$n^\star$ is rarely known exactly, but it has usable proxies. For binary classification with the 0-1 loss, $\mathcal{P}$ the set of all distributions (agnostic learning) and $\mathcal{H}$ of VC dimension d, $n^\star = \Theta\big((d + \log(1/\delta))/\varepsilon^2\big)$ [Shalev-Shwartz and Ben-David 2014], and data-dependent versions replace d with Rademacher complexity [Bartlett and Mendelson 2002]. For logistic and survival prediction models, the criteria of Riley et al. give closed forms. They are not sample complexities in the sense of Definition 2.1 but regression-specific surrogates, which give the size at which a model with p parameters meets targets on shrinkage, optimism and the precision of the intercept; we use them as proxies for $n^\star$ when the task is a clinical prediction model, and σ then inherits the error of the proxy. Their first criterion, which targets a global shrinkage factor S (typically 0.9) for a model with p candidate parameters and anticipated Cox-Snell R^2_{CS}, is

$$n^\star \;\ge\; \frac{p}{(S-1)\,\ln\!\big(1 - R^2_{\mathrm{CS}}/S\big)} \qquad \text{[Riley et al. 2019b].} \tag{1}$$

Either way, $n^\star$ grows with model complexity and with the precision demanded, and for binary outcomes it is driven by the number of events rather than the number of patients.

2.3 Effective sample size

Clinical records are not i.i.d., not all labeled, not all correctly labeled, and not drawn from the population where the model will be used. Each of these reduces the information a record carries.

Definition 2.2 (Effective sample size). Let $\mathcal{D}$ contain n records, and fix a target quantity $\psi(P)$, such as a model parameter or the risk of a fixed model, with an estimator $\hat{\psi}$ that is efficient under i.i.d. sampling from P. The effective sample size $n^{\mathrm{eff}}(\mathcal{D})$ is the number of i.i.d., correctly labeled records drawn from P for which $\hat{\psi}$ would have the same variance as the best estimator of $\psi(P)$ that can be computed from $\mathcal{D}$. To first order, when the mechanisms below act independently,

$$n^{\mathrm{eff}} \;=\; n \cdot \underbrace{\frac{1}{\mathrm{DEFF}}}_{\text{clustering}} \cdot \underbrace{\rho_L}_{\text{labels}} \cdot \underbrace{(1-2\eta)^2}_{\text{label noise}} \cdot \underbrace{\rho_S}_{\text{shift}}, \tag{2}$$

where $\mathrm{DEFF} = 1 + (\bar{m} - 1)\,\mathrm{ICC}$ is the design effect for $\bar{m}$ records per patient with intra-class correlation ICC [Kish 1965]; ρ_L is the fraction of records with labels missing completely at random; $\eta < 1/2$ is a symmetric label-noise rate, whose effect on sample complexity scales as $(1-2\eta)^{-2}$ [Angluin and Laird 1988] (for the prevalence of a binary label the exact factor is $(1-2\eta)^2\,\pi(1-\pi)/\tilde{\pi}(1-\tilde{\pi}) \le$

$(1-2\eta)^2$, where $\tilde{\pi}$ is the prevalence of the noisy labels); and $\rho_S = (\sum_i w_i)^2/(n\sum_i w_i^2)$ is Kish's efficiency of the importance weights $w_i = p_P(x_i)/p_{\mathcal{D}}(x_i)$ needed to transport the sample to P [Kish 1965]. The weights exist only if P is absolutely continuous with respect to the distribution that generated $\mathcal{D}$; where P puts mass on inputs that $\mathcal{D}$ cannot contain, no weighting recovers the information, and the effective sample size for that region is zero. For a binary outcome with prevalence π, the information about the decision boundary is controlled by the minority class, $n_{\text{ev}}^{\text{eff}} = n^{\text{eff}} \cdot \min(\pi, 1-\pi)$.

Equation (2) is deliberately simple. Each factor is a standard, estimable quantity, and each can be replaced by a more refined estimator (for instance, the fraction of missing information under multiple imputation) without changing the structure. The factors multiply because each is a ratio of variances under an independent mechanism; when mechanisms interact, the product is an approximation. None of the factors is new, and Equation (2) should be read as a first-order heuristic: it is meant to compare datasets and to show where information is lost, not to replace a variance computed for a specific estimator. What it adds is a single place where the clinical and the learning-theoretic notions of sample size meet, relative to a task.

Definition 2.3 (Smallness index). The smallness index of a dataset $\mathcal{D}$ for task $\mathcal{T}$ is $\sigma_{\mathcal{T}}(\mathcal{D}) = n^{\star}_{\mathcal{T}}/n^{\text{eff}}(\mathcal{D})$. The dataset is *small for* $\mathcal{T}$ when $\sigma_{\mathcal{T}}(\mathcal{D}) > 1$. We call the regimes $\sigma \le 1$ *sufficient*, $1 < \sigma \le 10$ *strained*, $10 < \sigma \le 100$ *small*, and $\sigma > 100$ *extreme*.

The thresholds are conventions, chosen so that each regime corresponds to a qualitatively different strategy. In the strained regime, regularization, simpler models and careful validation usually close the gap. In the small regime, information has to be imported from outside the dataset. In the extreme regime, only strong priors or pooling at a scale beyond any single institution will help.

Example 2.4 (An ICU deterioration model). A hospital has $n = 2{,}000$ ICU stays from patients with on average $\bar{m} = 1.6$ stays each and ICC $= 0.5$, so DEFF $= 1.3$. Seventy percent of stays have an adjudicated outcome, adjudication disagrees with a reference standard in 10% of cases (which we treat as a symmetric noise rate $\eta = 0.1$), and the case mix differs from the deployment population with Kish efficiency 0.6. By Equation (2), $n^{\text{eff}} \approx 414$, and at an outcome prevalence of 8% the dataset contains about 33 effective events. For a model with $p = 20$ parameters and an anticipated R^2_{CS} of 15% of its maximum, Equation (1) gives $n^{\star} \approx 2{,}707$. Hence $\sigma \approx 6.5$: a dataset that looks moderate is strained, close to small. Figure 2 shows the decomposition.

2.4 Siloed smallness

The definitions so far concern one dataset. Healthcare adds a second level: data that would be sufficient together are held separately.

Definition 2.5 (Siloed smallness). Let $\mathcal{D}_1, \ldots, \mathcal{D}_K$ be held by K custodians and let $\mathcal{D}_\cup$ be their union. The task $\mathcal{T}$ is *siloed-small* if $\sigma_{\mathcal{T}}(\mathcal{D}_k) > 1$ for every k while $\sigma_{\mathcal{T}}(\mathcal{D}_\cup) \le 1$, where $n^{\text{eff}}(\mathcal{D}_\cup)$ is computed after accounting for the heterogeneity across custodians (the shift factor ρ_S of Equation (2) applied to the mixture). The ratio $\Gamma = \min_k \sigma_{\mathcal{T}}(\mathcal{D}_k)/\sigma_{\mathcal{T}}(\mathcal{D}_\cup)$ is the *custodial gap*.

A siloed-small task can be solved in principle without collecting a single new patient; the obstacle is access. A task that is small even for $\mathcal{D}_\cup$ is *intrinsically small*, and no amount of collaboration among the existing custodians will solve it. Rare disease work often sits between the two. The distinction matters because the remedies are different, and because heterogeneity between custodians can make $n^{\text{eff}}(\mathcal{D}_\cup)$ much smaller than $\sum_k n^{\text{eff}}(\mathcal{D}_k)$. Section 8 makes this precise.

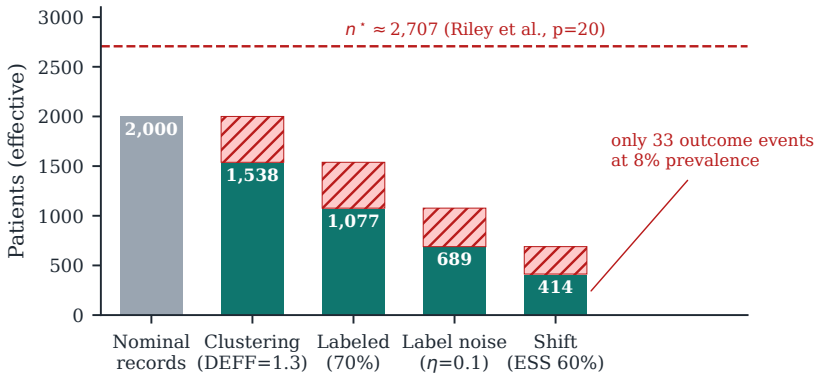


Fig. 2. Deflation of a nominal sample of 2,000 ICU stays into an effective sample size (Example 2.4). Hatched segments show the information lost to each mechanism of Equation (2). The dashed line is the sample size required by the first criterion of Riley et al. [2019b] for a 20-parameter model at 8% prevalence.

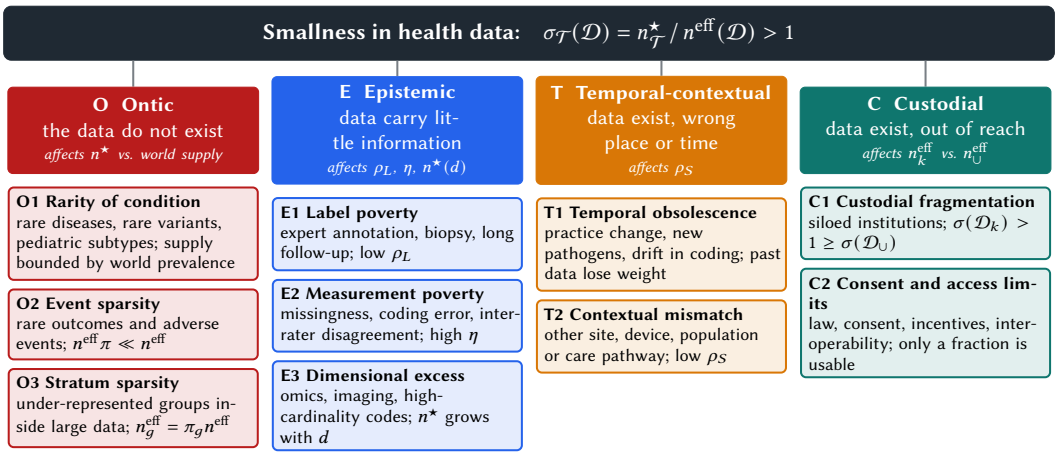


Fig. 3. The scarcity axis of the Scarcity-Provenance taxonomy. Families are defined by the term of the smallness index they degrade; the ten classes are the forms these take in healthcare.

2.5 Why healthcare is a special case

Small data exist in every field, but five properties make healthcare unusual. *Stakes are asymmetric*: errors fall on patients, so the tolerance ε is small and the required $n^{\star}$ is large. *Labels are expensive and delayed*: ground truth often needs specialist review, biopsy, or months of follow-up [Willemink et al. 2020]. *Heterogeneity is structural*: devices, coding practices, referral patterns and populations differ across sites in ways that models detect and exploit [Finlayson et al. 2021; Gichoya et al. 2022; Zech et al. 2018]. *Re-identification risk is high*: Rocher et al. estimate that 99.98% of Americans would be correctly re-identified in any dataset using 15 demographic attributes [Rocher et al. 2019], which undermines de-identification as a basis for sharing [Price and Cohen 2019; Sweeney 2002]. And *subgroups matter*: performance in a minority subgroup is a clinical and ethical requirement, not a secondary metric [Chen et al. 2021b; Obermeyer et al. 2019]. Each subgroup has its own, smaller n^{eff}.

From measurement to diagnosis. The smallness index tells us *how* small a dataset is for a task. It does not yet tell us *why*, and the why matters: a low ρ_L, a small π and a large custodial gap all raise σ, but each calls for a different response. The next section uses the structure of σ itself to classify the causes.

3 THE SCARCITY AXIS: WHERE THE DEFICIT COMES FROM

The first axis of our taxonomy answers the question *why is this dataset small?* Existing classifications tend to mix causes (rare disease), symptoms (class imbalance) and remedies (few-shot learning). We instead classify scarcity by the term of the smallness index it degrades. This makes the classes operational, because each one points to a specific quantity that can be estimated. They are not mutually exclusive. Rarity of condition (O1) and event sparsity (O2) both lower the effective number of events, contextual mismatch (T2) and custodial fragmentation (C1) usually arrive together, and a single cohort can be label-poor and high-dimensional at once. A dataset therefore has a *profile* over the ten classes, and that profile is more informative than any single label. This is also how we code the literature in Section 5: a work can target several classes.

3.1 Ontic scarcity (O): the data do not exist

Ontic scarcity is a property of the world, not of the dataset. More effort spent on collection or access will not remove it, only reduce it.

O1, rarity of condition. For a disease with worldwide prevalence q, the total supply of cases is bounded by qN_{world}, and in practice by the fraction diagnosed and recorded. When $n^\star$ approaches that bound, the task is intrinsically small in the sense of Definition 2.5. This is the situation of many of the conditions cataloged by Orphanet [Nguengang Wakap et al. 2020], and the reason machine learning in rare diseases relies so heavily on transfer, knowledge and international pooling [Banerjee et al. 2023; Schaefer et al. 2020].

O2, event sparsity. The condition may be common, but the outcome of interest is rare: in-hospital cardiac arrest, a severe adverse drug reaction, a post-operative complication. The number of patients is large but the effective number of events $n^{\text{eff}}\pi$ is small, which is what drives the sample-size criteria [Peduzzi et al. 1996; Riley et al. 2019b]. Oversampling or reweighting changes the decision threshold, not the information.

O3, stratum sparsity. Within a large dataset, a subgroup g with share π_g has $n_g^{\text{eff}} = \pi_g n^{\text{eff}}$. Clinically meaningful subgroups (by ancestry, sex, age band, comorbidity or disease subtype) are often small even when the whole is not. Failures concentrated in such strata have been called hidden stratification [Oakden-Rayner et al. 2020], and the related fairness failures are well documented [Chen et al. 2021b; Obermeyer et al. 2019].

3.2 Epistemic scarcity (E): the data carry little information

Here the patients exist and are recorded, but each record says less about the target than it could.

E1, label poverty. Unlabeled images, notes and signals are cheap, but labels need specialist time, invasive confirmation, or follow-up long enough for the outcome to occur [Willemink et al. 2020]. Label poverty acts through ρ_L. It is the scarcity class for which semi-supervised, self-supervised and weakly supervised learning were developed [Cheplygina et al. 2019; Tajbakhsh et al. 2020].

E2, measurement poverty. Missing values that are not missing at random, billing codes that reflect reimbursement more than pathology, and disagreement between raters all reduce the information per record. In Equation (2) this acts through η and through the missing-information fraction.

E3, dimensional excess. Genomic, transcriptomic and imaging data have $d \gg n$. In this case $n^\star$ grows with d, often faster than any realistic cohort, unless strong structural assumptions such

Table 2. Scarcity classes, the quantity that measures each, and the characteristic failure when it is ignored.

Class	What to estimate	Typical failure if ignored
O1 Rarity of condition	world supply qN_{world} vs. $n^{\star}$	optimistic results from a single centre that do not replicate
O2 Event sparsity	effective events $n^{\text{eff}}\pi$	overfitting, miscalibrated risks [Riley et al. 2019b]
O3 Stratum sparsity	n_g^{eff} for each relevant group g	hidden stratification, subgroup harm [Oakden-Rayner et al. 2020]
E1 Label poverty	labeled fraction ρ_L, label delay	models trained only on the easy, labeled cases
E2 Measurement poverty	noise rate η, missingness mechanism	learning the documentation process, not the disease
E3 Dimensional excess	d/n^{eff}, intrinsic dimension	spurious signatures, unstable feature selection
T1 Temporal obsolescence	time-sliced performance	silent decay after deployment
T2 Contextual mismatch	Kish efficiency ρ_S, site-held-out error	shortcut learning, external failure [Zech et al. 2018]
C1 Custodial fragmentation	custodial gap Γ	many under-powered single-site models
C2 Consent and access limits	usable fraction per purpose	selection bias in the usable subset

as sparsity are imposed [Bühlmann and van de Geer 2011; Tibshirani 1996]. Dimensional excess differs from the other classes in that smallness can be created by the choice of representation, and therefore partly removed by changing it.

3.3 Temporal-contextual scarcity (T): the data are about somewhere else

T1, temporal obsolescence. Treatment guidelines, coding systems, assays and pathogens change. Data collected before a change are partly irrelevant afterwards, so the usable window is shorter than the archive. The early COVID-19 literature is an extreme case [Roberts et al. 2021; Wynants et al. 2020].

T2, contextual mismatch. A model trained at one site, scanner or population meets a different distribution at deployment [Finlayson et al. 2021; Koh et al. 2021]. In Equation (2) this is the factor ρ_S, which can be close to zero when supports barely overlap. Learning theory makes the same point through the divergence term in domain adaptation bounds [Ben-David et al. 2010]. External validation studies show the effect directly [Debray et al. 2015; Wong et al. 2021; Zech et al. 2018].

3.4 Custodial scarcity (C): the data are out of reach

C1, custodial fragmentation. The information exists and is relevant, but is distributed across institutions that do not pool it. This is the silo problem formalized in Definition 2.5, and the setting of Section 8.

C2, consent and access limits. Even inside one institution, only part of the data can be used for a given purpose, because of consent scope, legal basis, governance review, or simply the absence of interoperable extraction [Cohen and Mello 2018; European Parliament and Council of the European Union 2016; Mandel et al. 2016]. Common data models such as OMOP and the OHDSI network [Hripcsak et al. 2015; Overhage et al. 2012] and the FAIR principles [Wilkinson et al. 2016] attack the interoperability side of C2.

Using the scarcity profile. Table 2 summarizes the ten classes with the quantity to estimate for each and the characteristic failure when it is ignored. We suggest that studies report this profile alongside n^{eff} and σ. It takes a few lines and would make results across small-data papers far easier to compare.

From diagnosis to remedy. A scarcity profile identifies which factor of σ is degraded, but not how to repair it. Every repair has to bring in information about the target from somewhere, and the possible sources are limited. The second axis of the taxonomy, introduced next, classifies remedies by that source.

4 THE PROVENANCE AXIS: WHERE THE MISSING INFORMATION COMES FROM

The second axis of the taxonomy classifies remedies, and it is the axis along which the rest of the paper is organized. Existing surveys group methods by technique ("generative models", "transfer learning", "federated learning"). Technique is the wrong primitive for small data, because two techniques that look different can draw on the same source of information, and two that look alike can draw on different ones. A GAN trained on the local cohort and a GAN pretrained on a million public images are both "synthetic data", but only the second brings anything new. We therefore classify remedies by the *provenance* of the information they add.

4.1 A conservation principle

Let θ denote the unknown quantity we want to learn: a parameter, a regression function, or a decision rule for the target population. Let $\mathcal{D}$ be the local data and let Z denote any external resource used by a method, such as a pretrained network, a knowledge base, or a published risk score.

Proposition 4.1 (No free lunch for re-expression). *Let $\tilde{\mathcal{D}} = g(\mathcal{D}, Z, U)$ be any dataset produced by a (possibly randomized) procedure g, where U is randomness independent of $(\theta, \mathcal{D}, Z)$. Then*

$$I(\theta; \mathcal{D}, \tilde{\mathcal{D}}) \;\le\; I(\theta; \mathcal{D}, Z).$$

In particular, if Z is independent of $(\theta, \mathcal{D})$, or absent, then $I(\theta; \mathcal{D}, \tilde{\mathcal{D}}) = I(\theta; \mathcal{D})$. Moreover, for any loss L and any estimator $\hat{\theta}(\mathcal{D}, \tilde{\mathcal{D}})$, the randomized estimator $(\mathcal{D}, Z, U) \mapsto \hat{\theta}(\mathcal{D}, g(\mathcal{D}, Z, U))$ has the same risk at every value of θ. Hence the minimax and Bayes risks attainable from $(\mathcal{D}, \tilde{\mathcal{D}})$ are no smaller than those attainable from $(\mathcal{D}, Z)$ with randomized estimators, and, for a loss convex in the estimate, no smaller than those attainable from $(\mathcal{D}, Z)$ without randomization.

Proof. Since U is independent of $(\theta, \mathcal{D}, Z)$ and $\tilde{\mathcal{D}}$ is a function of $(\mathcal{D}, Z, U)$, the chain $\theta \to (\mathcal{D}, Z) \to (\mathcal{D}, Z, \tilde{\mathcal{D}})$ is Markov. By the data processing inequality [Cover and Thomas 2006], $I(\theta; \mathcal{D}, Z, \tilde{\mathcal{D}}) \le I(\theta; \mathcal{D}, Z)$, and by the chain rule $I(\theta; \mathcal{D}, \tilde{\mathcal{D}}) \le I(\theta; \mathcal{D}, Z, \tilde{\mathcal{D}})$. For the second claim, $I(\theta; \mathcal{D}, Z) = I(\theta; \mathcal{D}) + I(\theta; Z \mid \mathcal{D})$ and $I(\theta; Z \mid \mathcal{D}) \le I(\theta, \mathcal{D}; Z) = 0$ when Z is independent of $(\theta, \mathcal{D})$. The reverse inequality $I(\theta; \mathcal{D}, \tilde{\mathcal{D}}) \ge I(\theta; \mathcal{D})$ holds trivially. For the risk statement, the two estimators coincide as functions of $(\mathcal{D}, Z, U)$, so their risks agree for every θ, and the set of estimators based on $(\mathcal{D}, \tilde{\mathcal{D}})$ is contained in the set of randomized estimators based on $(\mathcal{D}, Z)$. For a convex loss, replacing a randomized estimator $\delta(\mathcal{D}, Z, U)$ by $\mathbb{E}_U\, \delta(\mathcal{D}, Z, U)$ does not increase the risk at any θ, by Jensen's inequality. □

The proposition is elementary, but its consequences are not widely appreciated in the applied literature. The information statement treats θ as random; the risk statement holds for every fixed θ and every loss, and does not rely on lower bounds such as Fano's inequality. Together they say that no augmentation, oversampling or synthetic generation can lower the minimax or Bayes risk

for θ below what $(\mathcal{D}, Z)$ allows. Re-expression can still help in practice, and often does, for three reasons that the proposition makes explicit: the generator or augmentation family carries prior knowledge Z (rotation invariance in histology, anatomical plausibility in a pretrained generator); the downstream learner may use the re-expressed data more efficiently, which is a computational rather than an informational gain; or the synthetic data are used for a purpose other than learning θ, such as software testing. What re-expression cannot do is replace the missing patients.

4.2 Six provenance channels

Write the posterior for the parameter of site k as

$$p(\theta_k \mid \mathcal{D}_k, \mathcal{S}) \;\propto\; \underbrace{p(\mathcal{D}_k \mid \theta_k)}_{\text{likelihood: local data}} \;\cdot\; \underbrace{p(\theta_k \mid \mathcal{S})}_{\text{prior: everything else}} \;, \tag{3}$$

where $\mathcal{S}$ denotes all information other than the local labeled sample, and the local data are assumed to depend on $\mathcal{S}$ only through θ_k. Every remedy for small data does one of two things: it enlarges the likelihood term by adding observations that bear on θ_k, or it sharpens the prior term by bringing in information from $\mathcal{S}$. The six channels below exhaust the possible sources by construction, because they answer three questions about any input to a method other than $\mathcal{D}_k$. Is it data? If not, it is knowledge (P3). If it is data, was it newly acquired for the task? If so, it is procurement (P5). If it is existing data, does it carry labels for the same task and population? Labeled data for the same task held by other custodians is pooling (P1); data from a different task or population, labeled or not, is proxy (P2); unlabeled inputs from the target population are P4. A method whose only input is $\mathcal{D}_k$ itself, transformed, is re-expression (P6). Figure 4 shows the six channels.

P1 Pooling. Labeled data for the same task from other custodians: data sharing, consortia, distributed statistics, federated and decentralized learning. Information crosses a custodial boundary, so pooling carries a privacy cost, and heterogeneity between custodians limits what it can deliver.

P2 Proxy. Data from related but different tasks or populations: transfer learning, multi-task and meta-learning, self-supervised pretraining on external data, foundation models. The cost is relevance: the gain depends on how close the proxy is to the target [Ben-David et al. 2010; Tripuraneni et al. 2020].

P3 Prior. Non-data knowledge: physiology, pharmacology, clinical guidelines, ontologies, causal structure, published effect sizes, expert elicitation. It enters as constraints, priors or architecture.

P4 Unlabeled. Unlabeled data from the target population: semi-supervised and self-supervised learning, weak supervision. Unlabeled inputs are informative about θ only when the input distribution and the conditional of the label share parameters, which theory links to the causal direction of the task [Schölkopf et al. 2012].

P5 Procurement. New labeled observations obtained by deliberate action: prospective collection, active learning, targeted annotation. This is the only channel that increases the local likelihood term with fresh, relevant information.

P6 Re-expression. Transformations of existing data: augmentation, resampling, synthetic data, imputation. By Proposition 4.1 it adds no information beyond $(\mathcal{D}, Z)$; whatever it contributes comes from the prior knowledge in Z.

Two attributes of each channel matter for system design. The first is whether information crosses a custodial boundary, which determines the privacy cost: only P1 crosses one by necessity, and P2 does so when the proxy data belong to another custodian. The second is the source of bias: heterogeneity for P1, relevance for P2, misspecification for P3, the cluster or manifold assumption for P4, selection for P5 and generator bias for P6.

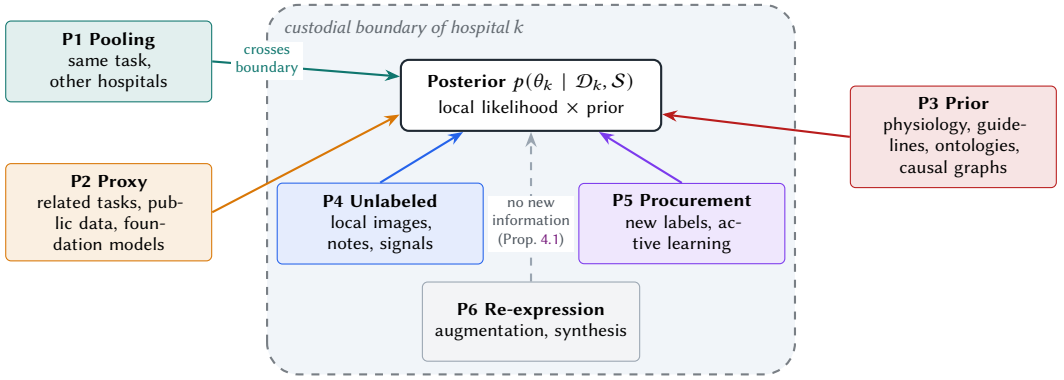


Fig. 4. The provenance axis. Each remedy draws information from one of six channels. Only pooling necessarily crosses the custodial boundary; only procurement adds fresh labeled observations from the target; re-expression adds no information about θ_k beyond what the local data and the generator already contain.

4.3 Matching scarcity to provenance

The two axes together give a matrix (Figure 5) that answers a practical question: given the scarcity profile of a problem, which channels can in principle help? The entries are our judgment, informed by the mechanisms of Section 4 and by the literature, graded from none to strong. They are claims about potential, not measurements, and Section 7 sets them against where the coded corpus has actually worked (Figure 8). Three patterns stand out. Ontic scarcity (O1 to O3) is addressed most strongly by pooling at a scale beyond the local site and by priors; procurement helps with event and stratum sparsity but is bounded by the world supply of cases for rare conditions. Custodial scarcity (C1, C2) is addressed by pooling far more than by any other channel, which is why privacy-preserving pooling is the central technical problem for siloed healthcare. And re-expression is weak against every class, which follows from Proposition 4.1 and matches the evidence.

The matrix is a set of claims about what can work in principle. The next three sections test it against what has actually been built and measured: Section 5 describes how we assembled and coded the literature, Section 6 reviews the methods channel by channel, and Section 7 asks what the reported results, and the corpus as a whole, do and do not support.

5 REVIEW METHODOLOGY

The taxonomy gives us a vocabulary. To use it as a survey instrument, we need a body of literature coded in that vocabulary and an honest account of how it was assembled. This section provides both, following the guidelines of Kitchenham and Charters [2007] for literature reviews and of Wohlin [2014] for snowballing, and using PRISMA 2020 [Page et al. 2021] as the reporting target.

5.1 Research questions

The review addresses five questions, each answered in a specific part of the paper.

RQ1 How can smallness be defined so that it is task-relative and measurable? (Section 2)

RQ2 Which remedies exist for each scarcity class, and through which provenance channel do they import information? (Section 6)

RQ3 At what level are these remedies supported by evidence: theory, a single dataset, or data held by several institutions? (Section 7)

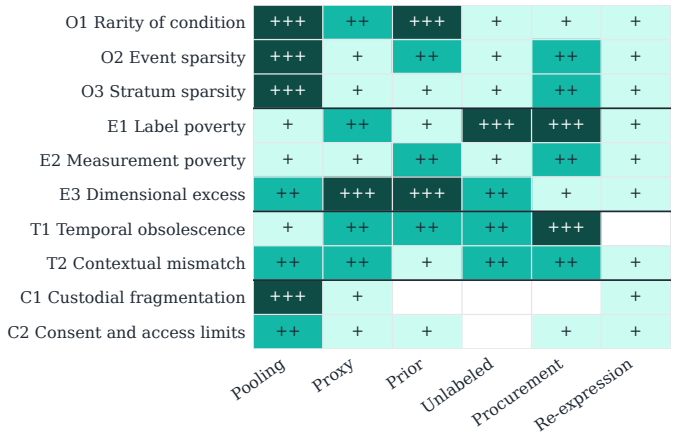


Fig. 5. The Scarcity-Provenance matrix. Rows are the ten scarcity classes of Figure 3; columns are the six provenance channels of Figure 4. Entries grade how strongly a channel can address a class (blank: not at all; + weak; ++ moderate; + + + strong). Grades are the authors' judgment of potential; observed coverage in the coded corpus is shown in Figure 8.

RQ4 Which combinations of scarcity class and channel that the taxonomy deems promising are under-studied? (Section 7)

RQ5 Which approach fits a sparse network of hospitals that cannot share records and has no trusted coordinator? (Section 8)

5.2 Corpus construction

The corpus was assembled in two stages. The *seed* set contained the foundational work for each provenance channel, the existing surveys listed in Section 1, and the meta-research on clinical prediction models and medical imaging AI. The *expansion* stage applied backward and forward snowballing [Wohlin 2014] from the seed set, with the inclusion criteria below, until new candidates stopped adding channel and modality combinations not yet covered. Every record was checked against the publisher's page or a bibliographic index, and records whose details we could not confirm were dropped.

A work was included if it (I1) proposes, evaluates or reviews a way of learning from limited, fragmented or poorly labeled data, and either addresses health data directly or is the method that health applications build on; or (I2) documents, in health data, a failure or risk caused by scarcity, by distribution shift or by sharing. Works were excluded if they (X1) are textbooks, regulations or reporting guidelines; (X2) concern scaling with abundant data; or (X3) provide general learning theory not tied to a specific channel. Excluded works are still cited where they support an argument. Of the 329 works identified in this way, 39 were excluded as background (X1: 18, X2: 5, X3: 16), which leaves a coded corpus of 290 works published between 1956 and 2025, 204 of them since 2019.

5.3 Coding scheme

Each work was coded on the five attributes of Table 3. Channels and scarcity classes are those of Sections 3 and 4. A work has exactly one primary channel, the one through which most of its information gain is claimed, and at most one secondary channel; 48 of the 257 works that propose

Table 3. Coding scheme applied to every work in the corpus.

Attribute	Values	Rule
Primary channel	P1 to P6, none	the channel through which most of the claimed information gain enters
Secondary channel	P1 to P6, empty	a second channel that is essential to the method, not incidental
Scarcity classes	O1 to C2, multi-label	the classes the work states it targets or documents, judged from its setting
Modality	imaging, EHR, tabular, text, omics, signals, multimodal, general	the data the work is evaluated on; general for methods not specific to health
Contribution type	method, theory, study, multi-site, resource, review, evidence	multi-site only if data come from two or more independent institutions

or study a remedy have one. Works that document a problem without proposing a remedy (33) are coded with the channel "none". Scarcity classes are multi-label and were assigned from the problem the work states it addresses and from its evaluation setting, not from what the method could do in principle. Contribution type records the primary contribution. A work is coded as a multi-site study only when its main contribution is training or validating on data held by two or more independent institutions; method papers that also report an external validation remain methods, and simulated partitions of a single dataset count as single-dataset evidence. The full coding is listed in the online supplement and released with the paper as a CSV file together with the scripts that produce every count and figure in Section 7.

5.4 Systematic search

The snowballed corpus was complemented by a systematic database search, executed on 8 October 2026 and reported following PRISMA 2020 [Page et al. 2021] (Figure 6). Its purpose is to test the corpus, not to replace it: we ask how much of the corpus a reproducible search recovers, and whether the coverage profile and the gaps of Section 7 hold in the wider literature.

Sources and query. PubMed was searched through the NCBI E-utilities, and medRxiv and the arXiv records indexed by Europe PMC through the Europe PMC API, from 2010 to the search date, in titles and abstracts. Europe PMC indexes only the life-science categories of arXiv, so computer-science preprints that are not also published in an indexed venue are not covered. Scopus, Web of Science, IEEE Xplore and the ACM Digital Library require subscriptions or API keys and were not searched; the scripts accept their exports, which are deduplicated and screened with the rest. The query, adapted to each source's syntax (both spellings replace the inner wildcard of decentrali*ed, which neither source supports), was:

```
("small data" OR "small sample*" OR "limited data" OR "data scarcity" OR "few-shot"
OR "low-resource" OR "rare disease*" OR "label-efficient" OR "data-efficient" OR
federated OR decentrali*ed OR "privacy-preserving") AND (health* OR clinical OR
medic* OR hospital* OR patient*) AND ("machine learning" OR "deep learning" OR
"transfer learning" OR "synthetic data" OR "data augmentation" OR "active learning"
OR "semi-supervised" OR "self-supervised" OR "foundation model*" OR "hierarchical
model*")
```

Screening. Records were deduplicated by DOI and normalized title, and titles and abstracts were screened against criteria I1, I2 and X1 to X3 by explicit rules, so that the screening can be rerun exactly. The rules exclude records whose only scarcity term refers to a resource-limited setting rather than to scarce data, to a sample-size limitation stated in passing, or to decentralized trials or care; records that are guidelines or statements; and records with no learning method. A single

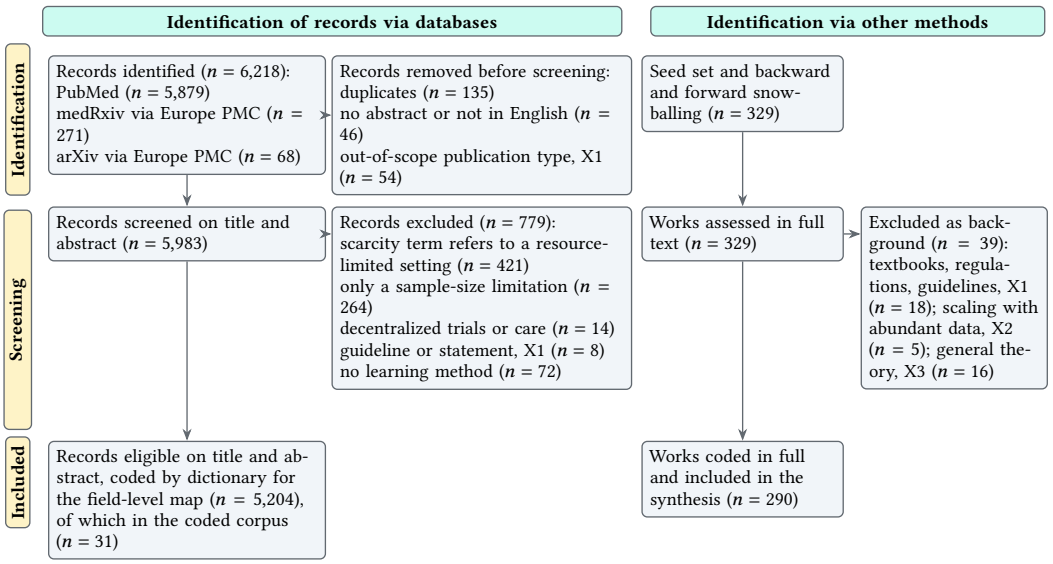


Fig. 6. PRISMA 2020 flow diagram. Left: the database search; its eligible records are coded by dictionary for the field-level map of Section 5.4 and are not part of the coded synthesis. Right: the seed set and snowballing of Section 5, which give the 290 works coded in full and analyzed in Sections 6 and 7.

reviewer checked the rules on 200 records, drawn in two random samples from both sides of the decision. Two rules that misfired in these checks were removed, and the final rules reached a precision of 63% and a negative predictive value of 94% against the reviewer's judgments. The screening is therefore deliberately sensitive: about a third of the eligible records are false positives, which the field-level analysis below tolerates but a synthesis would not. Eligible records were not assessed in full text.

How much of the corpus the search finds. Of the 201 corpus works that were published since 2010 and evaluated on health data, 144 are indexed in the searched sources, and the query retrieved 31 of them (22%). The missed works are mostly evidence of type I2, such as studies of external validation failure, hidden stratification or sample size, which describe a scarcity problem without the remedy vocabulary of the query, and methods that report a gain on a small dataset without naming scarcity. Broadening the vocabulary does not solve this: a version of the query with 39 additional scarcity terms and 16 additional method terms returned about 76,000 PubMed records and still found only about half of the corpus. This is the pattern that Greenhalgh and Peacock [2005] found for complex evidence, where protocol-driven searches located about a third of the relevant sources and snowballing most of the rest, and it is why the coded synthesis rests on the snowballed corpus while the search serves as a check on it.

The field-level map. The 5,204 eligible records were coded with dictionaries that encode the definitions of Sections 3 and 4. On the 147 corpus works with an abstract in Europe PMC, the dictionary coder agreed with the manual primary channel in 72% of cases (Cohen's $\kappa = 0.65$), and recovered 63% of the manually assigned scarcity classes. In the field, pooling is the most frequent primary channel (34%), followed by proxy (20%) and re-expression (12%); prior (3%), unlabeled (5%) and procurement (1%) are rare, and 26% of records match no channel. Pooling holds a similar share of the corpus (102 of 290 works), so its dominance is a property of the literature and not only of our

selection. Cell counts of the two maps are moderately correlated (Spearman $\rho = 0.55$), and 7 of the 10 gap cells of Figure 8 also fall in the bottom third of the field-level map: the temporal-obsolescence row and procurement for measurement poverty are as empty in the wider literature as in the corpus. Three gaps are not confirmed. Dimensional excess addressed by pooling is well populated in the field, mostly by federated and privacy-preserving genomic studies, and dimensional excess addressed by unlabeled data moderately so; both are under-covered by the corpus rather than by the literature. Stratum sparsity addressed by pooling also appears well populated, but most of its records match only the generic term bias, and the records that explicitly borrow strength across strata are almost all trial designs, such as basket trials and Bayesian subgroup analyses, which are outside the scope of this review (Section 1). Finally, 4,086 of the eligible records were published in 2023 or later, which confirms that the decline after 2022 in Figure 7 reflects the corpus and not the field.

5.5 Threats to validity

Four limitations qualify everything in the next two sections. First, *the coded corpus is curated, not exhaustive.* It was built to cover every cell of the taxonomy and to support the argument of Section 8, and counts describe the corpus, not the field. The systematic search of Section 5.4 bounds this limitation: it confirms the dominance of pooling and seven of the ten gaps, and it identifies three cells and the most recent years as under-covered by the corpus. Second, *coding was done by a single coder*, the first author, and inter-rater agreement has not been measured; the agreement with an independent dictionary coder ($\kappa = 0.65$) is a check of consistency, not a substitute. Third, the primary channel is a judgment about where a method's gain comes from, and some works, foundation models trained with self-supervision for example, sit between two channels. Fourth, the search covered PubMed, medRxiv and the life-science part of arXiv but not the subscription databases or the computer-science part of arXiv, and its eligible records were screened by rules checked on a sample rather than read in full, so the field-level map is a check on proportions, not a second corpus.

6 EXISTING SOLUTIONS, ORGANIZED BY PROVENANCE

We now review solution families in the order of the provenance channels of Section 4. For each family we describe the mechanism, the scarcity classes of Figure 3 it addresses, the healthcare evidence, and the conditions under which it fails. Pooling receives the most space, because it is the only strong channel against custodial scarcity and therefore the foundation of Section 8. Table 4 gives a compact overview. By primary channel, the corpus holds 102 pooling, 60 proxy, 25 prior, 23 unlabeled, 15 procurement and 32 re-expression works, plus 33 that document a problem without proposing a remedy; the online supplement lists and codes every one of them.

6.1 P1 Pooling: bringing the same task from other custodians

Centralized pooling and common data models. The simplest form of pooling is a shared repository. Public datasets such as MIMIC-III [Johnson et al. 2016] have been central to the field precisely because they removed custodial scarcity for one population. Distributed networks built on common data models, such as OHDSI on the OMOP model [Hripcsak et al. 2015; Overhage et al. 2012], keep data local but harmonize their structure so that the same analysis can be run everywhere. Harmonization reduces C2 scarcity but does not by itself allow joint model fitting.

Distributed statistics. Long before federated learning, biostatistics developed ways to fit models across sites without moving records. Meta-analysis combines site-level estimates, and random-effects meta-analysis [DerSimonian and Laird 1986] explicitly models between-site heterogeneity,

a point we return to in Section 8. Individual participant data meta-analysis is more powerful but needs record-level access [Riley et al. 2010]. Distributed regression exchanges sufficient statistics or gradients: GLORE fits logistic regression across sites by exchanging Newton-Raphson summaries [Wu et al. 2012]; DataSHIELD wraps such procedures in disclosure controls [Gaye et al. 2014]; ODAL and related one-shot algorithms use surrogate likelihoods to obtain near-pooled estimates with one round of communication [Duan et al. 2020a; Jordan et al. 2019]; and the Personal Health Train infrastructure sends analyses to data stations rather than data to analyses [Beyan et al. 2020]. Jochems et al. [2016] demonstrated a distributed model across hospitals in radiotherapy without any data leaving them. These methods are well understood, often exact, and auditable. Their limitation is model class: they work best for low-dimensional parametric models.

Federated learning. Federated averaging [McMahan et al. 2017] trains a shared model by alternating local gradient steps with server-side averaging. Healthcare adopted it quickly [Kaissis et al. 2020; Nguyen et al. 2022; Rieke et al. 2020; Xu et al. 2021]. Early work on institutional cyclical training [Chang et al. 2018] and on brain tumor segmentation showed that federated models could reach 99% of the quality of a model trained on centrally pooled data [Sheller et al. 2020]. EXAM, trained across 20 institutions to predict oxygen needs of COVID-19 patients, improved average AUC by 16% and generalisability by 38% over single-site models [Dayan et al. 2021]. The largest study to date, across 71 sites and 6,314 glioblastoma patients, improved delineation of the surgically targetable tumor by 33% over a model trained on public data [Pati et al. 2022]. FLamby provides seven cross-silo healthcare benchmarks with natural splits, which makes such claims testable [Ogier du Terrail et al. 2022].

The central difficulty is heterogeneity. When site distributions differ, local updates drift apart and the averaged model can be worse than local ones [Hsieh et al. 2020; Zhao et al. 2018]. Remedies include proximal regularization (FedProx) [Li et al. 2020c], control variates (SCAFFOLD) [Karimireddy et al. 2020], site-specific normalization (FedBN) [Li et al. 2021b], frequency-space augmentation for domain generalization (FedDG) [Liu et al. 2021], minimax objectives that protect the worst-off site [Mohri et al. 2019], and ensemble distillation [Lin et al. 2020]. Medical versions of these ideas harmonize amplitude spectra and local drift across scanners [Jiang et al. 2022], close the gap between federated and centralized segmentation on unseen sites [Xu et al. 2022], adapt across acquisition sites in multi-site fMRI [Li et al. 2020a], and combine self-supervised pretraining with federated fine-tuning to tolerate heterogeneous, sparsely labeled sites [Yan et al. 2023]. Federated transfer learning has also been applied to wearable sensor data [Chen et al. 2020b], and federated tensor factorization and patient-similarity learning to EHR phenotyping [Kim et al. 2017; Lee et al. 2018].

Clinical deployments. Beyond the landmark studies, the corpus contains a steady line of real multi-institution work. In structured EHR data, federated models have predicted hospitalizations from records held by separate custodians [Brisimi et al. 2018], used patient clustering to cope with heterogeneous intensive care populations [Huang et al. 2019], and predicted COVID-19 mortality across the hospitals of one health system [Vaid et al. 2021]. In imaging, federated training has been run across institutions for prostate segmentation [Sarma et al. 2021], breast density classification [Roth et al. 2020], COVID-19 diagnosis on CT [Bai et al. 2021; Dou et al. 2021], and prediction of response to neoadjuvant chemotherapy in triple-negative breast cancer from whole-slide images [Ogier du Terrail et al. 2023]; swarm learning has been applied to histopathology [Saldanha et al. 2022], and weakly supervised federated learning to gigapixel pathology [Lu et al. 2022]. The Personal Health Train trained a model on more than 20,000 lung cancer patients without moving their data [Deist et al. 2020], building on earlier distributed survival models in non-small cell lung cancer [Jochems et al. 2017]. At network scale, the OHDSI LEGEND study compared first-line antihypertensive classes across millions of patients in observational databases from several

countries [Suchard et al. 2019], using a standardized framework for patient-level prediction [Reps et al. 2018], and web services made distributed logistic and Cox regression available to clinical researchers [Jiang et al. 2013; Lu et al. 2015], with one-shot algorithms extending the approach to time-to-event outcomes [Duan et al. 2020b]. Most other pooling papers in the corpus, including several influential ones, evaluate on a single dataset partitioned to simulate sites [Guo et al. 2021; Li et al. 2019; Sheller et al. 2019]. That distinction matters in Section 7.

Personalized federated learning. A more radical response is to stop looking for one model. Federated multi-task learning [Smith et al. 2017], meta-learning based personalization [Fallah et al. 2020], Moreau-envelope regularization [T. Dinh et al. 2020], Ditto [Li et al. 2021a], shared representations with local heads (FedRep) [Collins et al. 2021], mixtures of distributions [Marfoq et al. 2021], attention-weighted collaboration among similar silos [Huang et al. 2021a] and clustered federated learning [Ghosh et al. 2020] all learn a model per site while sharing statistical strength. Tan et al. [2023] survey the area. Theory on shared representations explains why this works when sites share a low-dimensional structure [Baxter 2000; Du et al. 2021; Maurer et al. 2016; Tripuraneni et al. 2020].

Serverless and decentralized learning. Federated learning in its standard form assumes a central server that all sites trust to aggregate. Many hospital networks lack such an entity, or cannot agree on who should play it. Decentralized SGD replaces the server with neighbour averaging over a communication graph, and can match centralized rates when the graph mixes well [Koloskova et al. 2020; Lian et al. 2017; Nedić and Ozdaglar 2009]. Gossip algorithms make this robust to asynchrony [Boyd et al. 2006; Xiao and Boyd 2004]. Personalized decentralized learning over a similarity graph was introduced by Vanhaesebrouck et al. [2017] and extended with differential privacy by Bellet et al. [2018]. Cyffers and Bellet showed that decentralization itself amplifies privacy, because each party sees only what reaches it through the graph [Cyffers and Bellet 2022]. In healthcare, Swarm Learning used a blockchain to coordinate peer-to-peer training without a central server across four clinical use cases [Warnat-Herresthal et al. 2021], and ProxyFL shares only differentially private proxy models among peers [Kalra et al. 2023]. Split learning partitions the network itself between parties [Gupta and Raskar 2018; Vepakomma et al. 2018], and vertical federated learning addresses the case where sites hold different features for the same patients [Yang et al. 2019]. Peer-to-peer training has been shown for brain segmentation [Roy et al. 2019] and tumor segmentation in hospital networks [Tedeschini et al. 2022], and split learning has been evaluated on clinical imaging tasks [Poirot et al. 2019]. These healthcare demonstrations are few and mostly simulated, a gap that Section 8 addresses.

Privacy technologies. Sharing parameters or gradients is not private by default. Gradients can be inverted to reconstruct training images [Geiping et al. 2020; Zhu et al. 2019], and trained models leak membership [Carlini et al. 2022; Shokri et al. 2017]. Differential privacy (DP) [Dwork et al. 2006; Dwork and Roth 2014] bounds what any output reveals about an individual and has been adapted to deep learning [Abadi et al. 2016; Mironov 2017] and to federated training at the user or site level [McMahan et al. 2018; Wei et al. 2020]. Secure aggregation reveals only the sum of updates [Bonawitz et al. 2017]; secure multi-party computation [Evans et al. 2018] and homomorphic encryption [Gentry 2009] allow computation on encrypted data, with multiparty homomorphic encryption demonstrated for federated analytics in precision medicine [Froelicher et al. 2021]. The costs are well known. DP degrades accuracy, more so for small datasets, high-dimensional models and under-represented groups [Bagdasaryan et al. 2019; Jayaraman and Evans 2019; Tramèr and Boneh 2021]. Cryptographic methods protect the computation but not what the final model reveals. Robust aggregation protects against malicious participants [Blanchard et al. 2017]. In healthcare,

end-to-end privacy-preserving training combining secure aggregation and DP was demonstrated on pediatric chest radiographs [Kaissis et al. 2021], DP has been studied for medical imaging [Ziller et al. 2021] and for federated pathology [Adnan et al. 2022] and EHR models [Choudhury et al. 2019], and cryptographic systems support distributed cohort exploration [Raisaro et al. 2019] and genome-wide association studies [Cho et al. 2018]. The threat is concrete. Gradient inversion has been demonstrated on medical federated training, with risk governed by batch size and the number of local steps [Hatamizadeh et al. 2023]; deep networks can re-identify patients from chest radiographs [Packhäuser et al. 2022]; face-recognition software can identify research participants from MRI [Schwarz et al. 2019]; and re-identification attacks on health data have a long record [El Emam et al. 2011].

6.2 P2 Proxy: borrowing from related tasks

Transfer learning. Pretraining on a large source and fine-tuning on a small target is the most widely used remedy [Pan and Yang 2010]. Its value in medical imaging is more nuanced than often assumed. Raghu et al. found that ImageNet pretraining gave little performance benefit on large medical imaging tasks and that much smaller architectures performed comparably, with the benefit concentrated in faster convergence and in the smallest data regimes [Raghu et al. 2019]. Domain adaptation theory gives the reason: the target risk is bounded by the source risk plus a divergence between domains and the error of the best joint hypothesis [Ben-David et al. 2010]. When the proxy is far from the target, these terms dominate. Earlier empirical studies on medical images found that fine-tuning a pretrained network performed at least as well as training from scratch and was more robust to reductions in training data [Shin et al. 2016; Tajbakhsh et al. 2016], and the specialist-level results on skin and retinal images were themselves fine-tuned ImageNet models trained on large labeled sets [Esteva et al. 2017; Gulshan et al. 2016]. Medical sources make the proxy closer: 3D pretraining on aggregated segmentation datasets [Chen et al. 2019], self-supervised Models Genesis [Zhou et al. 2021] and the RadImageNet database [Mei et al. 2022] all aim at this, and domain adaptation methods target the remaining shift directly [Guan and Liu 2022; Kamnitsas et al. 2017].

Self-supervised and foundation models. Self-supervised pretraining by contrastive learning [Chen et al. 2020a] or masked reconstruction [He et al. 2022] removes the need for source labels and allows pretraining on in-domain unlabeled data. Azizi et al. showed that self-supervised pretraining on unlabeled medical images improved top-1 accuracy by 6.7% on dermatology classification and mean AUC by 1.1% on chest X-ray classification over supervised ImageNet pretraining [Azizi et al. 2021]. RETFound, pretrained on 1.6 million unlabeled retinal images, adapted to ocular and systemic disease detection with fewer labels than comparison models [Zhou et al. 2023]. Vision-language pretraining [Radford et al. 2021; Wang et al. 2022], large language models that encode clinical knowledge [Singhal et al. 2023] or extract clinical information from few examples [Agrawal et al. 2022], and EHR foundation models [Rasmy et al. 2021; Steinberg et al. 2021] extend the idea across modalities; Moor et al. [2023] argue for generalist medical models, and Bommasani et al. [2021] discuss their risks. Medical image-text pretraining [Huang et al. 2021b; Zhang et al. 2022] and contrastive pretraining on chest radiographs [Sowrirajan et al. 2021] improve label efficiency, and a model trained only on radiographs paired with their reports detected several pathologies at expert level without explicit labels [Tiu et al. 2022]. Computational pathology has seen the fastest growth, with general-purpose slide and tile encoders [Chen et al. 2024; Xu et al. 2024], vision-language models [Lu et al. 2024], and a foundation model that reported clinical-grade detection of rare cancers [Vorontsov et al. 2024]; segmentation [Ma et al. 2024] and cancer imaging biomarkers in limited-data settings [Pai et al. 2024] follow the same pattern. For EHR, sequence models pretrained on large patient histories [Choi et al. 2016; Li et al. 2020b] and a health-system scale language model

trained on clinical notes [Jiang et al. 2023] transfer to many prediction tasks, although a review of EHR foundation models found evaluations narrow and rarely tied to clinical value [Wornow et al. 2023]. Clinical and biomedical language models [Alsentzer et al. 2019; Lee et al. 2020] and general large language models [Nori et al. 2023] bring textual knowledge, and transcriptomic foundation models pretrained on tens of millions of single cells transfer to settings with few samples [Cui et al. 2024; Theodoris et al. 2023]. Parameter-efficient fine-tuning [Hu et al. 2022] reduces the number of parameters to adapt, which lowers $n^\star$ directly.

Tabular data. Much of healthcare is tabular, where deep learning has a weaker record. Tree ensembles still outperform deep networks on typical medium-sized tabular benchmarks [Grinsztajn et al. 2022]. Two recent lines are directly about small data. TabPFN is a transformer trained only on synthetic datasets drawn from a prior over structural causal models, which performs Bayesian inference in a single forward pass [Hollmann et al. 2023]; its second version outperformed an ensemble of strong baselines tuned for four hours, in 2.8 seconds, on datasets of up to 10,000 samples [Hollmann et al. 2025]. In the language of our taxonomy, it converts a prior (P3) into a proxy (P2) by sampling. TabLLM serializes rows into text and uses a large language model's knowledge for few-shot classification [Hegselmann et al. 2023], drawing on prior knowledge absorbed during pretraining. Both point to a general principle: in the small regime, the quality of the prior matters more than the flexibility of the model.

Few-shot and meta-learning. Meta-learning learns an inductive bias from a distribution of related tasks, so that a new task can be learned from few examples [Baxter 2000; Finn et al. 2017; Hospedales et al. 2022; Snell et al. 2017; Wang et al. 2020]. Its assumptions are demanding: many related training tasks drawn from the same distribution as the target task. In healthcare that distribution is often what is missing. Medical applications exist for clinical risk prediction from limited EHR [Zhang et al. 2019], rare skin conditions [Mahajan et al. 2020; Prabhu et al. 2019] and volumetric segmentation from few annotated scans [Roy et al. 2020], and facial-phenotype models for rare genetic syndromes [Gurovich et al. 2019; Hsieh et al. 2022] show how a proxy trained on many syndromes can support conditions with only a handful of known patients.

6.3 P3 Prior: knowledge that is not data

Bayesian and hierarchical modeling. Bayesian inference makes the prior explicit [Gelman et al. 2013]. Hierarchical models are the canonical way to share strength across groups, and their frequentist counterpart, shrinkage, dominates separate estimation under squared error in the Gaussian model with three or more groups [Efron and Morris 1973; Stein 1956]. Penalized regression [Hastie et al. 2009; Tibshirani 1996] is the prior in disguise, and its use is recommended for small clinical prediction datasets, although it does not replace adequate sample size [Riley et al. 2020; van Smeden et al. 2019]. The clinical evidence is more cautious than the theory. Shrinkage improved small-sample prognostic models in early comparisons [Steyerberg et al. 2000], but later studies showed that it does not guarantee better performance and is least reliable precisely when samples are smallest, because the amount of shrinkage is itself estimated with large uncertainty [Riley et al. 2021; Van Calster et al. 2020]. This is the same problem that the blueprint of Section 8 faces when it estimates the heterogeneity τ^2, and Proposition 8.1 quantifies it.

Mechanistic and physics-informed models. When the system has known dynamics, as in pharmacokinetics, cardiovascular hemodynamics or tumor growth, embedding them as constraints or residuals dramatically reduces the data needed [Karniadakis et al. 2021; Raissi et al. 2019]. von Rueden et al. [2023] classify how knowledge can enter learning: through the training data, the

hypothesis set, the learning algorithm or the final hypothesis. Medical examples include physics-informed networks for cardiac activation mapping [Sahli Costabal et al. 2020] and arterial blood pressure [Kissas et al. 2020], biologically informed networks for sparse experimental data [Lagergren et al. 2020], and neural ordinary differential equations for pharmacokinetics [Lu et al. 2021a]. Human concept learning from one example, formalized as probabilistic program induction [Lake et al. 2015], is the limiting case of what a strong prior can do.

Ontologies and knowledge graphs. Medical codes are organized in hierarchies. GRAM uses these ontologies to give rare codes representations close to their ancestors, improving prediction for codes that appear only a few times [Choi et al. 2017a]. This directly targets the long tail created by E3 and O1. KAME and GAMENet extend the idea with knowledge-graph attention and drug-interaction graphs [Ma et al. 2018; Shang et al. 2019], health knowledge graphs can be learned from records [Rotmensch et al. 2017], and the Human Phenotype Ontology [Köhler et al. 2021] supplies the structured vocabulary that rare-disease models rely on.

Causal knowledge. Causal structure tells a model which relations should be stable across sites and time. Invariance-based learning [Arjovsky et al. 2019], causal representation learning [Pearl 2009; Schölkopf et al. 2021] and causal analysis of imaging datasets [Castro et al. 2020] target T1 and T2 scarcity by excluding features whose relation to the outcome is site-specific. Graphical models of the shift itself can identify which relations will transport [Subbaswamy et al. 2019]. The empirical record is sobering, however: under careful model selection, domain generalization methods did not consistently beat plain empirical risk minimization, either on general benchmarks [Gulrajani and Lopez-Paz 2021] or in clinical settings [Zhang et al. 2021].

6.4 P4 Unlabeled: information in the inputs

Semi-supervised methods such as consistency regularization with pseudo-labels [Sohn et al. 2020] use unlabeled inputs to shape the decision boundary. Cheplygina et al. [2019] review semi-supervised, multiple-instance and transfer learning in medical imaging, and Tajbakhsh et al. [2020] review solutions for imperfect segmentation datasets. Weak supervision builds noisy labels from heuristics, knowledge bases and existing records and combines them with a generative label model [Ratner et al. 2017]. As Proposition 4.1 and the causal analysis of Schölkopf et al. [2012] imply, these methods help when the input distribution carries information about the label mechanism. They are effective against E1 and much less so against O-type scarcity.

In medical image segmentation, semi-supervised learning has been developed through self-training [Bai et al. 2017], uncertainty-aware and transformation-consistent self-ensembling [Li et al. 2021c; Yu et al. 2019], and contrastive pretraining on the unlabeled part of the same dataset [Chaitanya et al. 2020; Taleb et al. 2020], with few-shot segmentation trained from superpixels without annotation [Ouyang et al. 2020]; Krishnan et al. [2022] review self-supervision in medicine. In computational pathology, attention-based multiple-instance learning [Ilse et al. 2018] made slide-level labels sufficient: a system trained on tens of thousands of slides with only diagnostic labels reached clinical-grade performance [Campanella et al. 2019], and CLAM made the approach data-efficient [Lu et al. 2021b]. Large chest radiograph datasets were labeled by mining radiology reports [Irvin et al. 2019; Wang et al. 2017], which trades label poverty (E1) for measurable label noise (E2). In EHR data, anchor-based phenotyping [Halpern et al. 2016], learning from noisy labels [Agarwal et al. 2016] and weak supervision of clinical text [Fries et al. 2019; Wang et al. 2019] play the same role.

6.5 P5 Procurement: acquiring the right new data

Procurement is the most neglected channel in machine learning papers and the most important one in clinical research. Sample size calculations [Riley et al. 2020] turn smallness into a design problem before data collection starts. Active learning chooses which examples to label next to maximize information [Settles 2009], and has been widely studied in medical imaging [Budd et al. 2021]. Representative methods select annotation regions by uncertainty and representativeness [Yang et al. 2017], use learned feature distances [Smailagic et al. 2018], or exploit Bayesian uncertainty [Gal et al. 2017; Nath et al. 2021]. Learning curves fitted on pilot data can predict how many labeled examples a target accuracy needs [Cho et al. 2015; Figueroa et al. 2012], and Balki et al. [2019] review sample-size methods for machine learning in imaging; for continuous outcomes the clinical criteria have their own form [Riley et al. 2019a]. Procurement is the smallest channel in our corpus (15 works), which is at odds with its importance. Data valuation methods such as Data Shapley [Ghorbani and Zou 2019] estimate which records or sources contribute most, which could guide both acquisition and the fair crediting of contributors in a consortium.

6.6 P6 Re-expression: augmentation and synthetic data

Data augmentation encodes invariances and is standard practice [Shorten and Khoshgoftaar 2019; Zhang et al. 2018]. Oversampling methods such as SMOTE [Chawla et al. 2002] interpolate minority examples. Deep generative models, from GANs [Goodfellow et al. 2014] to diffusion models [Ho et al. 2020; Kazerouni et al. 2023], can produce realistic medical images, and tabular and EHR generators include medGAN [Choi et al. 2017b] and CTGAN [Xu et al. 2019]. In a widely cited study, GAN-based augmentation improved liver lesion classification from 78.6% to 85.7% sensitivity and from 88.4% to 92.4% specificity on a dataset of 182 lesions [Frid-Adar et al. 2018]. Similar results have been reported for brain MRI synthesis used for both augmentation and anonymization [Shin et al. 2018], for segmentation [Bowles et al. 2018], for 3D generation with diffusion models [Khader et al. 2023], and for learned spatial and appearance transformations that make one-shot segmentation possible [Chaitanya et al. 2021; Zhao et al. 2019]; Yi et al. [2019] and Chlap et al. [2021] review the area. A large study found that diffusion-based augmentation improved fairness and robustness of medical classifiers under distribution shift [Ktena et al. 2024]; in our terms, the information came from the generator's training data, not from the act of sampling. For structured records, EHR generators [Baowaly et al. 2019; Zhang et al. 2020], anonymization-oriented synthesis [Beaulieu-Jones et al. 2019; Yoon et al. 2020], diffusion and language-model tabular generators [Borisov et al. 2023; Kotelnikov et al. 2023], and the rule-based Synthea simulator [Walonoski et al. 2018] have been proposed and evaluated [Goncalves et al. 2020]. Rebalancing deserves a warning: in clinical risk prediction, class imbalance corrections produced badly calibrated risks without improving discrimination [van den Goorbergh et al. 2022], despite their popularity [Fernández et al. 2018].

Synthetic data are also promoted as a privacy technology [Chen et al. 2021a; Jordon et al. 2022]. Here the evidence is sobering. Stadler et al. found that synthetic data either fail to protect against linkage and attribute inference or lose much of their utility, and do not provide a better privacy-utility trade-off than traditional anonymization [Stadler et al. 2022]. Formally private generators such as PATE-GAN [Jordon et al. 2019], built on the PATE framework [Papernot et al. 2017], provide guarantees at the cost of fidelity. And recursive training on generated data causes models to lose the tails of the distribution [Shumailov et al. 2024], which in healthcare are exactly the rare cases and minority groups that motivated the effort. Proposition 4.1 summarizes the position: synthetic data are a useful vehicle for information that already exists (in a pretrained generator, or in another site's data under a privacy guarantee), but not a source of it.

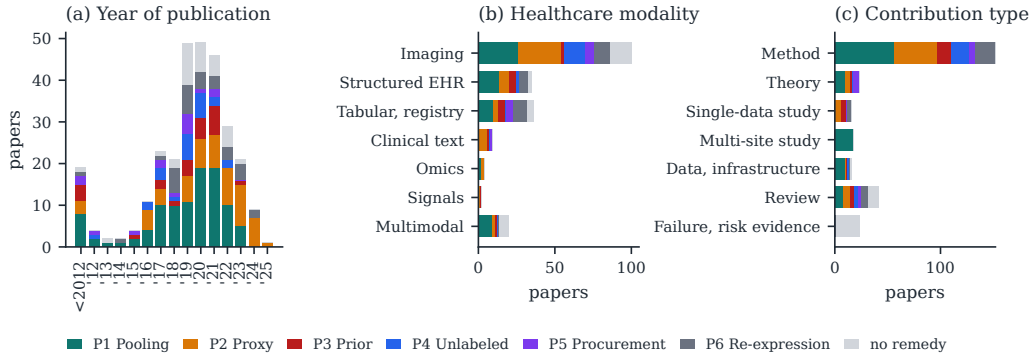


Fig. 7. Profile of the coded corpus (290 works), colored by primary provenance channel. (a) Year of publication; the drop after 2022 reflects indexing and citation lag in a curated corpus, not a trend. (b) Data modality, for the 206 works evaluated on health data. (c) Primary contribution type.

Taken together, the six channels cover a wide range of techniques, but the evidence behind them is uneven. Some channels have been tested at multi-site scale, others mainly on internal splits of a single dataset. The next section reads the reported results through the taxonomy to separate the two.

7 WHAT THE EVIDENCE SHOWS, AND WHAT IT DOES NOT

We read the evidence at two levels: the shape of the coded corpus, which shows where the literature has worked, and the quantitative findings it reports, which show what that work achieved. All counts below come from the coding of Section 5 and are subject to the threats to validity listed there.

7.1 Where the literature has worked

Figure 7 profiles the corpus. Two thirds of the 290 works (204) appeared in or after 2019. Of the 206 works evaluated on health data, almost half (100) use imaging, while structured EHR (35), tabular registries (36), clinical text (9), omics (4), physiological signals (2) and multimodal data (20) share the rest; the remaining 84 are general methods that health applications build on. Most works propose a method (152). Only 18 have a multi-institution study as their primary contribution, and every one of them is a pooling study. No proxy, prior, unlabeled, procurement or re-expression work in the corpus has a multi-institution study as its main result, although several report external validation alongside a method.

Figure 8 sets the coverage of each cell of the taxonomy against the judged potential of Figure 5. Coverage is very uneven. Label poverty (E1, 100 works) and custodial fragmentation (C1, 97 works) dominate, and they are served by different channels: E1 mainly by proxy (56) and unlabeled data (33), C1 almost only by pooling (93). Temporal obsolescence (T1) and measurement poverty (E2) are targeted by 8 works each, and stratum sparsity (O3) by 10, although these are the classes that the external-validation failures of Section 1 point to. Ten cells judged at least moderately promising contain two works or fewer; they are outlined in red. The systematic search of Section 5.4 confirms seven of them in the wider literature; the exceptions are dimensional excess addressed by pooling or by unlabeled data, which the corpus under-covers, and stratum sparsity addressed by pooling, where the wider literature borrows across strata mainly in trial design. Four of them lie in the T1 row and four in the procurement column. Channels are also rarely combined. Of the 257 remedy

Table 4. Representative solution families by provenance channel, with the scarcity classes they target, the main assumption, and the main limitation in healthcare.

Family	Representative work	Targets	Key assumption	Main limitation
P1 Pooling				
Common data models	OHDSI/OMOP [Hripcsak et al. 2015; Overhage et al. 2012]	C2	semantic harmonization is feasible	harmonization cost; no joint fitting by itself
Distributed statistics	GLORE [Wu et al. 2012], ODAL [Duan et al. 2020a], DataSHIELD [Gaye et al. 2014]	C1, O2	low-dimensional parametric model	limited model classes
Federated learning	FedAvg [McMahan et al. 2017], EXAM [Dayan et al. 2021], [Pati et al. 2022]	C1, O1, O2	trusted server; moderate heterogeneity	non-i.i.d. drift [Hsieh et al. 2020]; leakage without DP [Geiping et al. 2020]
Personalized FL	Ditto [Li et al. 2021a], FedRep [Collins et al. 2021], IFCA [Ghosh et al. 2020]	C1, T2	shared low-dim. structure	small sites may still overfit local heads
Decentralized learning	D-PSGD [Lian et al. 2017], Swarm [Warnat-Herresthal et al. 2021], ProxyFL [Kalra et al. 2023]	C1	connected graph	slow mixing on sparse graphs
P2 Proxy				
Transfer learning	[Pan and Yang 2010; Raghu et al. 2019]	E1, E3	source close to target	negative transfer; limited gain on large tasks
Self-supervised and foundation models	[Azizi et al. 2021; Moor et al. 2023; Zhou et al. 2023]	E1, E3, T2	pretraining data cover target	bias inherited from pretraining; compute
Tabular priors	TabPFN [Hollmann et al. 2025], TabLLM [Hegselmann et al. 2023]	E3, O2	synthetic or textual prior matches reality	limited size and feature range; opacity
Meta-learning	MAML [Finn et al. 2017], ProtoNets [Snell et al. 2017]	O1, E1	many related tasks available	task distribution itself scarce
P3 Prior				
Hierarchical Bayes, shrinkage	[Efron and Morris 1973; Gelman et al. 2013]	O1 to O3, C1	exchangeability across groups	misspecified hyperpriors
Physics-informed	[Karniadakis et al. 2021; Raissi et al. 2019]	O1, E3	known mechanism	mechanism often partial in medicine
Ontology-guided	GRAM [Choi et al. 2017a]	E3, O1	ontology reflects similarity	ontologies built for billing
Causal and invariant	IRM [Arjovsky et al. 2019], [Castro et al. 2020]	T1, T2	stable causal mechanisms	needs diverse environments
P4 Unlabeled, P5 Procurement				
Semi- and weak supervision	FixMatch [Sohn et al. 2020], Snorkel [Ratner et al. 2017]	E1	cluster or manifold assumption	confirmation bias
Active learning, design	[Budd et al. 2021; Riley et al. 2020; Settles 2009]	E1, O2	labeling oracle available	cold start; sampling bias
P6 Re-expression				
Augmentation, SMOTE	[Chawla et al. 2002; Shorten and Khoshgoftaar 2019]	E1, O2	invariances known	no new information (Proposition 4.1)
Synthetic data	[Frid-Adar et al. 2018; Jordon et al. 2019; Xu et al. 2019]	C1, C2, O2	generator faithful	privacy-utility trade-off [Stadler et al. 2022]; tail loss [Shumailov et al. 2024]

works, 48 have a secondary channel, mostly self-supervised pretraining that mixes proxy and unlabeled information (16) and personalized pooling that mixes pooling with a prior or a shared representation (11). No work in the corpus combines pooling with procurement.

7.2 What the reported results show

Table 5 collects quantitative results reported in the literature. They are not directly comparable, since tasks, metrics and baselines differ, and we report them as the original authors did. Read through the taxonomy and together with the corpus profile, they support six observations.

1. Pooling works when the custodial gap is real, and is the only channel tested across institutions. The largest gains in Table 5 come from federated pooling in settings that are siloed-small in the sense of Definition 2.5: rare tumors, a new disease, many modest sites. This is what the matrix in Figure 5

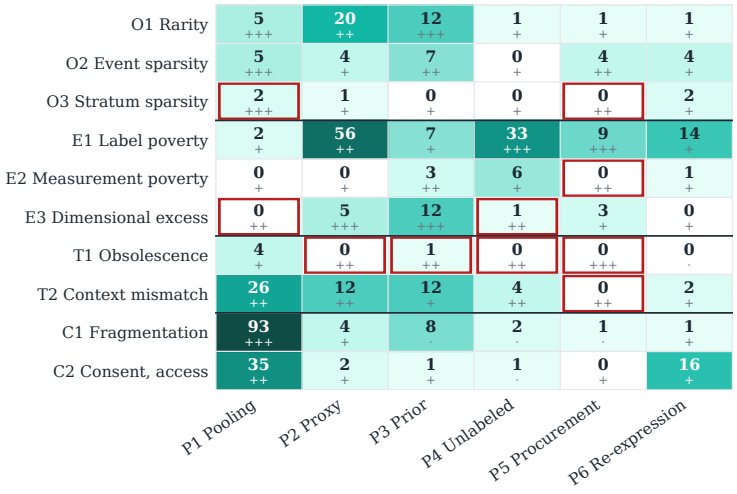


Fig. 8. Gap map. Each cell gives the number of corpus works that target a scarcity class (rows) through a channel, as primary or secondary channel (columns); shading is on a log scale. The small marks below each count repeat the judged potential of Figure 5 (+ weak to + + + strong, · none). Red outlines mark cells judged at least moderately promising that contain two works or fewer.

predicts. The gains are reported relative to single-site or public-data baselines, and are largest at the smallest sites. Pooling is also the only channel whose evidence includes multi-institution studies as primary results (18 of 102 pooling works); the other 84 evaluate on simulated partitions, public benchmarks or theory.

2. Proxy information helps in proportion to its relevance. Self-supervised pretraining on in-domain unlabeled data outperforms generic ImageNet transfer [Azizi et al. 2021; Raghu et al. 2019], and the strongest small-data results on tabular problems come from models whose prior was designed for small tabular problems [Hollmann et al. 2025]. Generic scale is less valuable than relevant structure. Foundation models are the fastest-growing part of the proxy channel, but their evaluation is still mostly on curated benchmarks [Wornow et al. 2023], so how much they reduce σ at a specific hospital is largely unmeasured.

3. Re-expression helps modestly and does not travel. Augmentation gains are real but usually reported on internal test sets, and the privacy case for synthetic data is weak [Stadler et al. 2022]. Rebalancing by resampling harms calibration in clinical risk models [van den Goorbergh et al. 2022]. Where generative augmentation does help under shift [Ktena et al. 2024], the generator was trained on much larger data, so the gain comes through the proxy channel. All of this follows from Proposition 4.1.

4. Evaluation is itself a small-data problem. The most consistent finding of the meta-research is that small studies produce over-optimistic and unstable estimates. Cross-validation on 100 samples has error bars near ±10% [Varoquaux 2018]; reported accuracy in some imaging applications falls as studies get larger [Varoquaux and Cheplygina 2022]; under-powered studies inflate effect sizes generally [Button et al. 2013; Ioannidis 2005]; and most COVID-19 models could not be used [Roberts et al. 2021; Wynants et al. 2020]. Among imaging AI studies, only 6% of 516 performed external validation [Kim et al. 2019]; systematic comparisons with clinicians found few studies with

Table 5. Selected quantitative findings, as reported by the original authors.

Channel	Study	Reported finding
P1	Sheller et al. [2020]	Federated training of brain tumor segmentation reached 99% of the model quality of centrally pooled data.
P1	Dayan et al. [2021]	Across 20 institutions, federated EXAM improved average AUC by 16% and generalisability by 38% over single-site models.
P1	Pati et al. [2022]	71 sites, 6,314 patients: 33% better delineation of the surgically targetable tumor and 23% of the whole tumor than a publicly trained model.
P2	Azizi et al. [2021]	Self-supervised pretraining: +6.7% top-1 accuracy in dermatology, +1.1% mean AUC on chest X-rays.
P2	Raghu et al. [2019]	ImageNet transfer gave little benefit on large medical imaging tasks; small models performed comparably.
P2, P3	Hollmann et al. [2025]	TabPFN outperformed baselines tuned for 4 h, in 2.8 s, on datasets up to 10,000 samples.
P6	Frid-Adar et al. [2018]	GAN augmentation on 182 liver lesions: sensitivity 78.6% to 85.7%, specificity 88.4% to 92.4%.
P6	Stadler et al. [2022]	Synthetic data gave no better privacy-utility trade-off than traditional anonymization.
T2	Wong et al. [2021]	A widely deployed sepsis model: external AUC 0.63, missing 67% of sepsis cases.
T2	Zech et al. [2018]	Pneumonia CNNs identified the source hospital; external performance was significantly lower in 3 of 5 comparisons.
O2, T1	Roberts et al. [2021]	Of 62 COVID-19 imaging models reviewed in detail, none was of potential clinical use.
all	Varoquaux [2018]	With about 100 samples, cross-validation error bars are around ±10%.
C	Rocher et al. [2019]	99.98% of Americans re-identifiable from 15 demographic attributes.
C	Bagdasaryan et al. [2019]	DP training reduces accuracy more for under-represented groups.

out-of-sample validation and frequent risk of bias [Liu et al. 2019; Nagendran et al. 2020]; externally validated algorithms often lose performance [Yu et al. 2022]; and in clinical prediction, machine learning showed no performance benefit over logistic regression in comparisons at low risk of bias [Christodoulou et al. 2019]. A method that "works on small data" has often only been evaluated on small data. External, site-held-out validation and reporting standards such as TRIPOD+AI [Collins et al. 2024] address this directly.

5. Privacy costs are paid by the smallest. DP noise is calibrated to sensitivity, which scales inversely with sample size, and its accuracy cost falls disproportionately on small datasets and minority groups [Bagdasaryan et al. 2019; Tramèr and Boneh 2021]. A privacy mechanism designed without attention to small sites can cancel the benefit of pooling for exactly the participants who need it. Section 8 designs around this.

6. Research effort follows convenient data, not severe scarcity. The corpus profile shows two dense regions, label poverty in imaging and custodial fragmentation through pooling, and three nearly empty rows: temporal obsolescence, measurement poverty and stratum sparsity. The first two are as empty in the systematic search as in the corpus. Procurement, which clinical research treats as the first remedy, is the smallest channel and is never combined with pooling. The empty cells of Figure 8 are not evidence that those remedies fail; they are untested, and they are where new work is most likely to change practice.

7.3 Limitations by channel

Pooling assumes a trusted coordinator, roughly exchangeable sites, and the governance to run a consortium; the first and third are often missing. Model updates leak information without formal protection [Geiping et al. 2020; Zhu et al. 2019], and formal protection costs accuracy. Heterogeneity can make a global model worse than a local one for some sites [Hsieh et al. 2020]. **Proxy** methods inherit the biases and blind spots of the source, including demographic shortcuts [Gichoya et al. 2022], and their benefit is hard to predict before trying. **Prior** knowledge is incomplete for most diseases and, when wrong, biases the model in a way that more local data corrects only slowly. **Unlabeled** data help only under assumptions on the data distribution that are rarely tested. **Procurement** is slow, expensive and subject to the same governance as any data collection. **Re-expression** cannot exceed the information in its inputs. Across all channels, uncertainty is under-reported: modern networks are poorly calibrated [Guo et al. 2017], and calibration is the first property to fail under small samples and shift.

What the gaps imply. The six observations and the limitations above point to the same place. Pooling is the only strong remedy for custodial scarcity (observation 1), but in its standard form it needs a trusted server, assumes away heterogeneity, and lets privacy costs fall on the smallest participants (observation 5). Proxy and prior information are most useful as a starting point rather than a substitute (observation 2), and re-expression cannot carry the load (observation 3). A method for siloed hospitals should therefore combine personalized pooling with privacy spent where it hurts least, on top of a public representation, and be evaluated honestly (observation 4). It should also leave room for the neglected channels, in particular procurement inside a consortium (observation 6). The next section derives such a method.

8 A BLUEPRINT FOR SPARSE NETWORKS OF SILOED HOSPITALS

We now turn to the setting that motivates this paper: a set of hospitals, each too small to learn a task reliably, that do not share patient data and are connected only by a sparse set of bilateral agreements. In the terms of Section 2, the task is siloed-small (Definition 2.5); in the terms of Section 3, its dominant scarcity is custodial fragmentation (C1), usually combined with event or stratum sparsity (O2, O3); and by Figure 5, the only channel that addresses it strongly is pooling. We ask what a theoretically grounded form of pooling looks like under the constraints of this setting, derive it in a model that can be solved exactly, and then show how the other channels of Section 4 fit around it. This section gives the design argument and its main results; a companion paper [Simões et al. 2026] turns it into a method for realistic model heads, analyzes its privacy in full, and evaluates it against federated and decentralized baselines.

8.1 Problem statement

There are K hospitals, the vertices of an undirected connected graph $\mathcal{G} = (V, E)$. An edge means that two hospitals have an agreement allowing them to exchange privatized model summaries; $\mathcal{G}$ is sparse, with $|E| = O(K)$, and there is no vertex that every hospital trusts. Hospital k holds n_k labeled records from P_k and wants a model that performs well on its own population. We require:

(R1) no patient-level record leaves its hospital;
(R2) every released quantity satisfies (ϵ, δ)-differential privacy with respect to the patients of the releasing hospital;
(R3) no central server: computation uses only messages along edges of $\mathcal{G}$;
(R4) the smallest hospitals benefit at least as much as the largest, in relative terms;
(R5) heterogeneity across hospitals is modeled rather than assumed away.

Requirements (R4) and (R5) exclude a single global model: by observation 5 of Section 7 and the non-i.i.d. results of Section 6.1, a global model serves the typical site, not the smallest or most atypical.

Why existing data-exchange agreements are not enough. Hospitals already exchange data under protocols, but those protocols do not remove the problem. Most of them serve primary use: a patient's record follows the patient to the next point of care. Training a model on the records of many hospitals is secondary use, which needs its own legal basis, ethics approval and data-access permit, and frequently excludes the export of patient-level records altogether [European Parliament and Council of the European Union 2016, 2025]. Where agreements for secondary use exist, they are typically bilateral, slow to negotiate and limited to a subset of hospitals and data, so the network they form is the sparse graph $\mathcal{G}$ assumed here: GHB is designed to run over existing agreements, not to replace them. Even where pooling is lawful, a central copy of identifiable records is not the least invasive option, given how easily such records are re-identified [Rocher et al. 2019], and data minimization favours releasing one privatized low-dimensional summary per hospital. Existing infrastructures that move analyses to the data rather than data to a centre, such as the OHDSI network [Hripcsak et al. 2015], DataSHIELD [Gaye et al. 2014] and the Personal Health Train [Beyan et al. 2020; Deist et al. 2020], are complementary: they provide the channels, and GHB specifies what to compute through them, how to combine the results and what it costs in privacy. Finally, even unrestricted pooling yields one model for all hospitals, which Section 8.7 shows to be worse than local estimation for atypical and large hospitals; the personalization of GHB is useful whether or not privacy binds. The converse also holds. When a network already has a trusted environment in which pooling is lawful, such as a national registry or a secure processing environment under the European Health Data Space, requirement (R3) is moot, and the hierarchical borrowing of Section 8.2 can run inside that environment without gossip or privatized releases.

8.2 Hierarchical borrowing with an imperfect prior

We analyze the core of the problem in the simplest model that keeps its structure. Each hospital has a parameter $\theta_k \in \mathbb{R}$ (one coordinate of a model head; the vector case is coordinatewise), and an asymptotically normal local estimate:

$$\theta_k = \mu + u_k, \quad u_k \sim \mathcal{N}(0, \tau^2); \qquad \hat{\theta}_k = \theta_k + e_k, \quad e_k \sim \mathcal{N}(0, v_k), \quad v_k = \sigma^2/n_k, \tag{4}$$

with all u_k, e_k independent. Here τ^2 is the between-hospital heterogeneity, the quantity that random-effects meta-analysis estimates [DerSimonian and Laird 1986], and σ^2 absorbs the inverse Fisher information per patient. This is the classical setting of empirical Bayes [Efron and Morris 1973; Gelman et al. 2013]. What is new here is that the population mean μ must be estimated privately and without a server, so hospital k only has access to some estimate $\tilde{\mu}$ with error.

Proposition 8.1 (Borrowing with an imperfect prior). *Let $\tilde{\mu}$ be any estimate of μ that is independent of (u_k, e_k) and has mean squared error $M = \mathbb{E}(\tilde{\mu} - \mu)^2$. Among estimators $\tilde{\theta}_k = \alpha\hat{\theta}_k + (1-\alpha)\tilde{\mu}$, the risk $R_k(\alpha) = \mathbb{E}(\tilde{\theta}_k - \theta_k)^2$ is minimized at*

$$\alpha_k^\star = \frac{\tau^2 + M}{\tau^2 + M + v_k}, \qquad R_k^\star = \frac{v_k\,(\tau^2 + M)}{v_k + \tau^2 + M} \;\le\; \min\{v_k,\ \tau^2 + M\}.$$

Equivalently, the effective sample size of hospital k becomes

$$n_k^{\text{eff}} = \frac{\sigma^2}{R_k^\star} = n_k + \underbrace{\frac{\sigma^2}{\tau^2 + M}}_{\text{borrowed patients } B}. \tag{5}$$

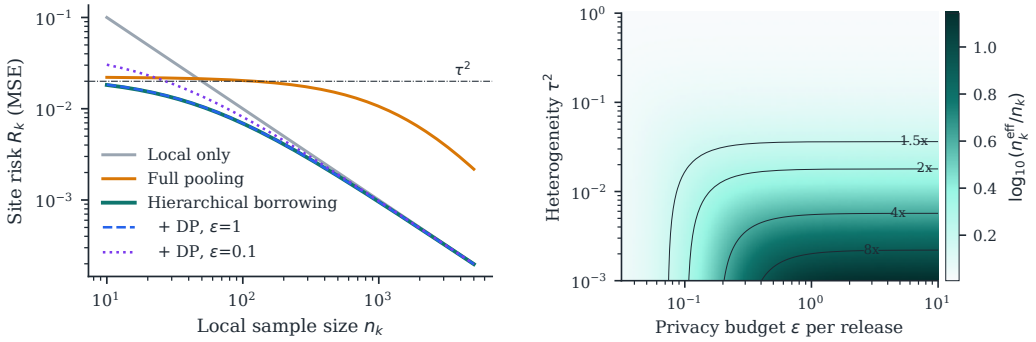


Fig. 9. Left: risk of hospital k as a function of its size n_k, with $K = 12$, other hospitals of 200 patients, $\sigma^2 = 1$, $\tau^2 = 0.02$ and $\delta = 10^{-5}$. Full pooling is excellent for small sites until it hits the heterogeneity floor τ^2, and then harms large ones; hierarchical borrowing (Proposition 8.1), with τ^2 and M known, is never worse than either in expected risk. Privacy at $\epsilon = 1$ is almost free; at $\epsilon = 0.1$ it reduces but does not remove the gain. Right: relative gain in effective sample size n_k^{eff}/n_k for a hospital with $n_k = 50$, as a function of heterogeneity and privacy budget, from Equations (5) and (6).

Proof. $\tilde{\theta}_k - \theta_k = \alpha e_k + (1-\alpha)(\tilde{\mu} - \mu) - (1-\alpha)u_k$. The three terms are mutually independent, and the first and last have zero mean, so the cross terms vanish; the middle term has second moment M. Hence $R_k(\alpha) = \alpha^2 v_k + (1-\alpha)^2(\tau^2 + M)$. This is a convex quadratic in α, minimized at $\alpha_k^\star$; substituting gives $R_k^\star$. The bound follows because $R_k^\star$ is half the harmonic mean of v_k and $\tau^2 + M$. Finally $\sigma^2/R_k^\star = \sigma^2/v_k + \sigma^2/(\tau^2 + M) = n_k + B$. □

Two remarks fix the scope of Proposition 8.1. First, the proof uses only first and second moments and independence, so it does not need the normality of Equation (4); normality matters only for reading $R_k^\star$ as a posterior variance. Second, R_k is an expectation over the site effect u_k as well as over the data: it is the risk of a hospital drawn from the population of hospitals, not of a particular one. For an unbiased $\tilde{\mu}$ and a given u_k, the risk is $\alpha^2 v_k + (1-\alpha)^2(M + u_k^2)$, which exceeds v_k when $|u_k|$ is large compared with τ. A genuinely atypical hospital can therefore lose by borrowing. This is why τ^2 is estimated from the data rather than assumed small, and why the structured prior of Section 8.4 lets similarity decide which hospitals borrow from which.

Equation (5) is the central quantity of the blueprint. Collaboration gives every hospital almost the same number B of *virtual patients*, regardless of its size; B depends on k only through the leave-one-out error M, which varies little across hospitals when there are many of them. A hospital with 30 patients and one with 3,000 both gain B; the first may double or triple its effective sample, the second barely notices. Requirement (R4) is therefore satisfied by construction. B is limited by heterogeneity: as $\tau^2 \to \infty$, $B \to 0$ and each hospital should act alone; as $\tau^2 \to 0$ with $M \to 0$, $B \to \infty$ and full pooling is optimal. Hierarchical borrowing interpolates between the two, using the data to decide where. With τ^2 and M known it never does worse than local estimation, which is not true of a global model (Figure 9); when both are estimated from the privatized releases the loss is small in simulation, but no longer zero (Section 8.7).

8.3 The cost of privacy

Proposition 8.1 holds for any estimate $\tilde{\mu}$. Requirements (R2) and (R3) constrain how that estimate can be obtained, and the next two results quantify what each constraint costs. Hospital k needs $\tilde{\mu}$ from the others, and each release must be private (R2). Note what does not need privatizing:

hospital k uses its own estimate $\hat{\theta}_k$ without noise, since that never leaves the hospital. Privacy affects only M.

Proposition 8.2 (Privatized population estimate). *Suppose $\hat{\theta}_j$ is the mean of per-patient contributions clipped to $[-C, C]$, so that replacing one patient changes $\hat{\theta}_j$ by at most $\Delta_j = 2C/n_j$, and that clipping introduces no bias. Each hospital j releases $\hat{\theta}_j + \xi_j$ with $\xi_j \sim \mathcal{N}(0, s_j^2)$ and $s_j = \Delta_j\sqrt{2\ln(1.25/\delta)}/\epsilon$, which is (ϵ, δ)-differentially private for $\epsilon \in (0, 1)$ by the Gaussian mechanism [Dwork and Roth 2014]. For the leave-one-out average $\tilde{\mu} = \frac{1}{K-1}\sum_{j\neq k}(\hat{\theta}_j + \xi_j)$,*

$$M = \frac{1}{(K-1)^2}\sum_{j\neq k}\Big(\tau^2 + \frac{\sigma^2}{n_j} + \frac{8C^2\ln(1.25/\delta)}{n_j^2\,\epsilon^2}\Big). \tag{6}$$

Proof. The terms $u_j + e_j + \xi_j$ are independent with zero mean and variances $\tau^2 + \sigma^2/n_j + s_j^2$, and $s_j^2 = 4C^2 \cdot 2\ln(1.25/\delta)/(n_j^2\epsilon^2)$. Independence from (u_k, e_k) holds because k is excluded. □

Three consequences follow. First, privacy noise is divided by $(K-1)$, so it vanishes as the network grows, while the local estimate remains noise-free. Second, the privacy term scales as n_j^{-2} while the sampling term scales as n_j^{-1}: privacy is cheap for large contributors and expensive for tiny ones. This is the formal version of observation 5 in Section 7. The remedy is to let small hospitals release jointly: with secure aggregation [Bonawitz et al. 2017] among neighbours, noise can be calibrated to the sensitivity of the sum, which is set by the total size of the group rather than by each member. Third, inverse-variance weighting of the releases, rather than equal weights, reduces M further and is what a practical implementation should use; Equation (6) is the simplest case.

Unknown heterogeneity and the scope of the guarantee. Proposition 8.1 takes τ^2 as known. It can be estimated from the releases by the method of moments of random-effects meta-analysis [DerSimonian and Laird 1986], which is post-processing; better, each hospital can integrate τ out over its posterior given the releases, weighting by the marginal likelihood of its own data, so that a hospital whose data conflict with the network borrows little. The privacy guarantee holds under explicit conditions: a patient held by several hospitals is protected at the composed level, site sizes are public, a vector head is released as one vector, estimators that are not clipped means need their own sensitivity, and repeated refits compose. For likelihood-based heads the natural release is the privatized score at the published model, inverted by public moment matching; a wrong anchor then makes the release noisier rather than biased. The companion paper [Simões et al. 2026] develops each of these points.

8.4 Computation on a sparse graph without a server

The leave-one-out average needs every hospital's release. Without a server, the hospitals compute it by gossip over $\mathcal{G}$ [Boyd et al. 2006; Xiao and Boyd 2004]. Let W be a symmetric, doubly stochastic matrix supported on the edges of $\mathcal{G}$, for instance Metropolis-Hastings weights $W_{kl} = 1/(1 + \max(d_k, d_l))$ for $(k, l) \in E$, where d_k is the degree of k. Starting from the privatized releases $x_j^0 = \hat{\theta}_j + \xi_j$, each round sets $x^{t+1} = Wx^t$.

Proposition 8.3 (Free post-processing and convergence). *(i) Every message exchanged during gossip is a function of the privatized releases only, so the whole gossip phase satisfies the same (ϵ, δ) guarantee as the initial release, with no additional privacy cost. (ii) Let $\lambda = \max\{|\lambda_2(W)|, |\lambda_K(W)|\}$. If $\mathcal{G}$ is connected and W has a positive diagonal, then $\lambda < 1$ and*

$$\big\|x^t - \bar{x}\mathbf{1}\big\|_2 \;\le\; \lambda^t\,\big\|x^0 - \bar{x}\mathbf{1}\big\|_2,$$

so that the estimate held by any hospital after t rounds has root mean squared error at most $\sqrt{M_{\text{all}}} + \lambda^t \sqrt{\mathbb{E}\|x^0 - \bar{x}\mathbf{1}\|_2^2}$ as an estimate of μ, where $M_{\text{all}} = K^{-2}\sum_j(\tau^2 + \sigma^2/n_j + s_j^2)$ is the mean squared error of the full average. (iii) Hospital k obtains its leave-one-out estimate as $\tilde{\mu}_k^t = (Kx_k^t - x_k^0)/(K-1)$. It converges to the leave-one-out average of Proposition 8.2, and differs from it by $K(x_k^t - \bar{x})/(K-1)$, whose root mean square is at most $\frac{K}{K-1}\lambda^t\sqrt{\mathbb{E}\|x^0 - \bar{x}\mathbf{1}\|_2^2}$.

Proof. (i) is the post-processing property of differential privacy [Dwork and Roth 2014]: the gossip iterates are a deterministic function of the released values, and each hospital's subsequent computations on its own data are never released. (ii) Since W is symmetric and doubly stochastic, $\mathbf{1}$ is an eigenvector with eigenvalue 1 and $x^t - \bar{x}\mathbf{1} = W^t(x^0 - \bar{x}\mathbf{1})$ lies in the orthogonal complement of $\mathbf{1}$, on which $\|W\|_2 = \lambda$. Connectivity and a positive diagonal make W irreducible and aperiodic, so $\lambda < 1$ [Xiao and Boyd 2004]. The error bound follows from the triangle inequality in L^2, since each coordinate deviation is bounded by the norm of the whole vector. (iii) Since $\sum_{j\neq k} x_j^0 = K\bar{x} - x_k^0$, the leave-one-out average is $(K\bar{x} - x_k^0)/(K-1)$, and subtracting it from $\tilde{\mu}_k^t$ leaves $K(x_k^t - \bar{x})/(K-1)$, which is bounded as in (ii). □

Part (iii) also bounds the one departure from Proposition 8.1 that gossip introduces: after finitely many rounds, $\tilde{\mu}_k^t$ still contains a fraction of hospital k's own release, so it is not exactly independent of (u_k, e_k). The residual dependence decays as λ^t, and a hospital should use $\tilde{\mu}_k^t$ only once λ^t is small; Section 8.7 shows what happens otherwise.

The number of rounds needed for accuracy γ is about $\log(1/\gamma)/(1-\lambda)$, so the spectral gap $1-\lambda$ of the collaboration graph determines the communication cost. The spectral gap is related to how well the graph is connected through Cheeger-type inequalities [Chung 1997]. Figure 10 shows this for 24 hospitals under four realistic topologies. A chain of bilateral agreements arranged in a ring mixes slowly; dense regional clusters joined by single bridges are not much better, because the bridges are bottlenecks; a few additional random or two-hop links triple the spectral gap, which after a hundred rounds lowers the consensus error by orders of magnitude. The practical lesson is that *which* agreements a network signs matters more than *how many*: adding a handful of long-range edges is worth more than densifying regions. Privacy amplification by decentralization [Cyffers and Bellet 2022] gives a further benefit on sparse graphs, because each hospital observes only what reaches it through its neighbours.

Structured heterogeneity. The exchangeable model (4) treats all other hospitals as equally relevant. When similarity is structured, for instance by region or case mix, the prior can follow a similarity graph with weights $a_{kl} \geq 0$, which may differ from the communication graph. A Gaussian Markov random field prior [Rue and Held 2005] with graph Laplacian L gives the maximum a posteriori problem

$$\min_{\boldsymbol{\theta}} \sum_k \frac{(\theta_k - \hat{\theta}_k)^2}{2v_k} + \frac{\beta}{2}\sum_{k<l} a_{kl}(\theta_k - \theta_l)^2 + \frac{\gamma}{2}\sum_k (\theta_k - \mu_0)^2, \tag{7}$$

whose solution satisfies the linear system $(V^{-1} + \beta L + \gamma I)\boldsymbol{\theta} = V^{-1}\hat{\boldsymbol{\theta}} + \gamma\mu_0\mathbf{1}$ with $V = \operatorname{diag}(v_k)$. The Jacobi iteration

$$\theta_k \leftarrow \frac{\hat{\theta}_k/v_k + \beta\sum_l a_{kl}\theta_l + \gamma\mu_0}{1/v_k + \beta d_k + \gamma}, \qquad d_k = \textstyle\sum_l a_{kl},$$

uses only neighbour values and converges for any starting point, because the system matrix is strictly diagonally dominant. This generalizes the personalized peer-to-peer objective of Vanhaesebrouck et al. [2017] with an explicit anchor μ_0, which can come from published knowledge (channel P3). To respect (R2), neighbours exchange smoothed values computed from privatized releases, and

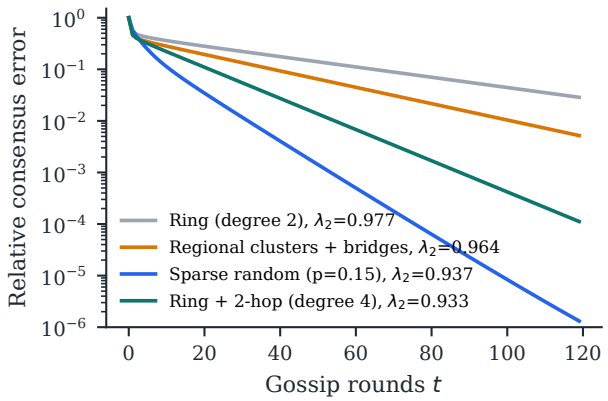


Fig. 10. Gossip consensus error for 24 hospitals on four sparse topologies with Metropolis-Hastings weights. Rates follow the second largest eigenvalue modulus λ of the gossip matrix (Proposition 8.3).

each hospital applies the final update with its own noise-free $\hat{\theta}_k$ locally without releasing it, so Proposition 8.3(i) applies.

8.5 From the Gaussian model to deep models

The analysis applies directly to parametric models, and in particular to the last layer of a network on top of a fixed representation. That observation suggests the architecture in Figure 11, which we call *Graph Hierarchical Borrowing* (GHB). It uses each provenance channel where it is strongest and keeps the privatized, shared part of the model small.

L0 Public representation (P2). A foundation encoder f_{ϕ_0} pretrained on public or licensed data for the modality [Moor et al. 2023; Rasmy et al. 2021; Zhou et al. 2023]. It is available to all hospitals without any privacy cost.

L1 Local self-supervised adaptation (P4). Each hospital adapts the encoder to its own unlabeled data with low-rank adapters [Hu et al. 2022] and a self-supervised objective [Chen et al. 2020a; He et al. 2022]. Adapters may be averaged by privatized gossip, but need not be.

L2 Hierarchical heads (P1). Each hospital fits a low-dimensional head $\theta_k \in \mathbb{R}^p$ on the adapted features, with an estimate of its covariance, and borrows strength by Propositions 8.1, 8.2 and 8.3 or by Equation (7).

L3 Knowledge anchor (P3). The hyperprior mean μ_0 comes from published models or clinical knowledge, so that even a hospital with no neighbours starts from a sensible place.

L4 Site calibration. Each hospital calibrates its predictions by conformal prediction on local held-out data [Angelopoulos and Bates 2023; Shafer and Vovk 2008], or by federated conformal prediction when the local calibration set is too small [Lu et al. 2023].

L5 Targeted procurement (P5). Posterior variances from L2 identify which patients or subgroups would be most informative to label next [Settles 2009].

Synthetic data (P6) is deliberately absent from the training path. It is useful for testing pipelines and for teaching, but by Proposition 4.1 it cannot add information, and its privacy case is weak [Stadler et al. 2022].

Two features of GHB deserve emphasis. Keeping the privatized component low-dimensional is not a convenience but a necessity: the norm of Gaussian-mechanism noise grows with $\sqrt{p}$, and

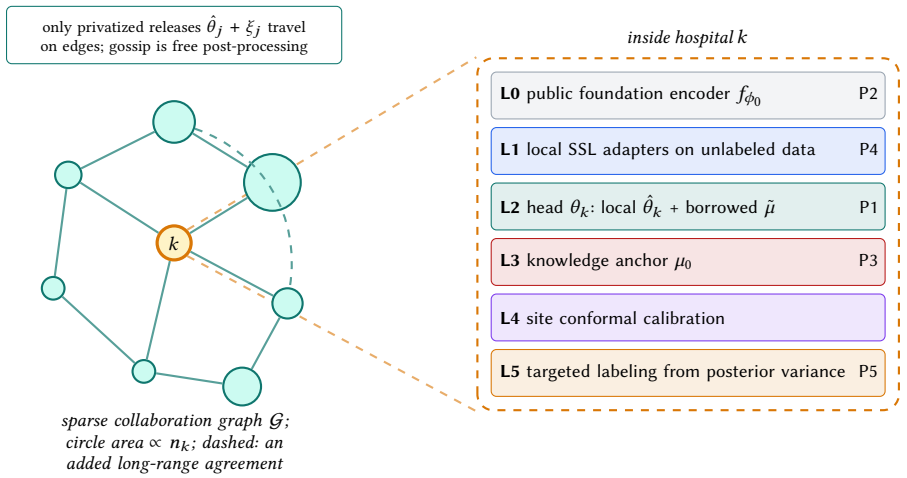


Fig. 11. Graph Hierarchical Borrowing (GHB). Left: hospitals of very different sizes connected by a sparse set of agreements, without a central server. Right: the model held by each hospital, with the provenance channel used at each layer. Only the low-dimensional head participates in privatized exchange.

private learning is far more accurate on good fixed features than end to end [Tramèr and Boneh 2021]. And the split between a public representation and a private head mirrors the theory of shared representations, in which a common representation reduces the per-task sample complexity to that of the head [Collins et al. 2021; Du et al. 2021; Maurer et al. 2016; Tripuraneni et al. 2020]. Only privatized summaries of the heads cross a hospital's boundary, and everything else is computed from them or stays local. The companion paper [Simões et al. 2026] gives the full protocol and its privacy boundary. There, the network releases two privatized sums over all of its patients, computed by secure summation without a server, so that the noise is calibrated to the whole network rather than to the smallest hospital. A lighter variant, GHB-lite, in which every hospital releases on its own, follows the analysis of this section most closely.

8.6 Why this is a principled default

Table 6 compares GHB with the alternatives on requirements (R1) to (R5). Centralized pooling violates (R1). Federated averaging violates (R3) and, under heterogeneity, (R4) and (R5). Personalized federated learning satisfies (R4) and (R5) but still needs a server. Existing decentralized approaches satisfy (R3); Swarm Learning does not provide formal differential privacy by default, while ProxyFL does but shares full proxy models. Synthetic data sharing satisfies (R1) and (R3) only at the cost of utility or privacy. Distributed meta-analysis is the closest classical relative and satisfies most requirements, but does not exploit learned representations or graph structure. The comparison is one of design properties, not of measured performance: the GHB row records what holds by construction under the assumptions of Propositions 8.1, 8.2 and 8.3, while the other rows describe each method in its standard form, and most of them can be extended to satisfy more requirements.

The argument for GHB rests on four results that are each well established, combined in a way that, to our knowledge, has not been set out for this setting. None of the four is new, and the case for GHB is that it is the simplest combination that meets all five requirements at once, not that it outperforms the alternatives; that has to be shown empirically. Separate estimation of three or more group means is inadmissible under squared error, and empirical Bayes shrinkage toward a common mean dominates it in the Gaussian model with equal variances [Efron and Morris 1973;

Table 6. Approaches for sparse, siloed hospital networks against requirements (R1) to (R5), as design properties of each method in its standard form. ✓ satisfied; ○ partially or optionally; ✗ not satisfied. Entries for GHB hold by construction under the assumptions of Section 8 and of the companion paper [Simões et al. 2026]; they have been checked in simulation and on real cancer networks (Section 8.7).

Approach	(R1) local data	(R2) formal DP	(R3) no server	(R4) small sites gain	(R5) heterogeneity
Centralized pooling	✗	✗	✗	○	○
Federated averaging [McMahan et al. 2017]	✓	○	✗	✗	✗
Personalized FL [Collins et al. 2021; Li et al. 2021a]	✓	○	✗	✓	✓
Swarm Learning [Warnat-Herresthal et al. 2021]	✓	✗	✓	○	✗
ProxyFL [Kalra et al. 2023]	✓	✓	✓	○	○
Synthetic data sharing [Jordon et al. 2019]	✓	○	✓	○	✗
Distributed meta-analysis [DerSimonian and Laird 1986; Duan et al. 2020a]	✓	○	○	✓	✓
GHB (this paper)	✓	✓	✓	✓	✓

Stein 1956]; Proposition 8.1 gives the best combination of a local and a population estimate in closed form when the population estimate is itself imperfect. Differential privacy composes and is closed under post-processing [Dwork and Roth 2014], which makes the gossip phase free (Proposition 8.3). Gossip converges at a rate set by the spectral gap [Boyd et al. 2006; Xiao and Boyd 2004], which turns network design into a measurable quantity. And shared representations reduce the problem to a low-dimensional one [Maurer et al. 2016; Tripuraneni et al. 2020], which is what keeps privacy noise small. Each component can be replaced by a stronger one, such as secure aggregation within regions, or nonlinear heads, without changing the architecture; integrating τ out over its posterior (Section 8.3) is one such replacement, and releasing only network-wide sums by secure summation, so that the privacy noise is calibrated to the whole network rather than to each hospital, is another, which the companion paper adopts after it proved decisive on real data.

8.7 What simulation and real data show

The propositions take τ^2 and M as known, the model as Gaussian and every participant as honest. The companion paper [Simões et al. 2026] tests what is lost when these assumptions fail, in eighteen Monte Carlo simulations whose main results Figure 12 summarizes. In a network of 24 hospitals of 16 to 479 patients, hierarchical borrowing with every unknown estimated from the privatized releases stays at or below local estimation for every hospital across four orders of magnitude of heterogeneity, while a single global model becomes up to 382 times worse for its worst hospital. Every hospital gains about $B \approx 46$ virtual patients, which multiplies the effective sample size of hospitals with 60 patients or fewer by 2.28 and of those with 300 or more by 1.08. With logistic-regression heads, against federated averaging with fine-tuning, decentralized DP-SGD and clustered federated learning at the same privacy budget, with their hyperparameters tuned on the true loss, the borrowing hospital's worst case stays within a factor 1.06 of its own local error in every private setting except the most heterogeneous, while the iterative methods leave some hospital, usually a large one, 1.39 to 2.96 times worse off. Adversarial simulations show the limits: a small hospital

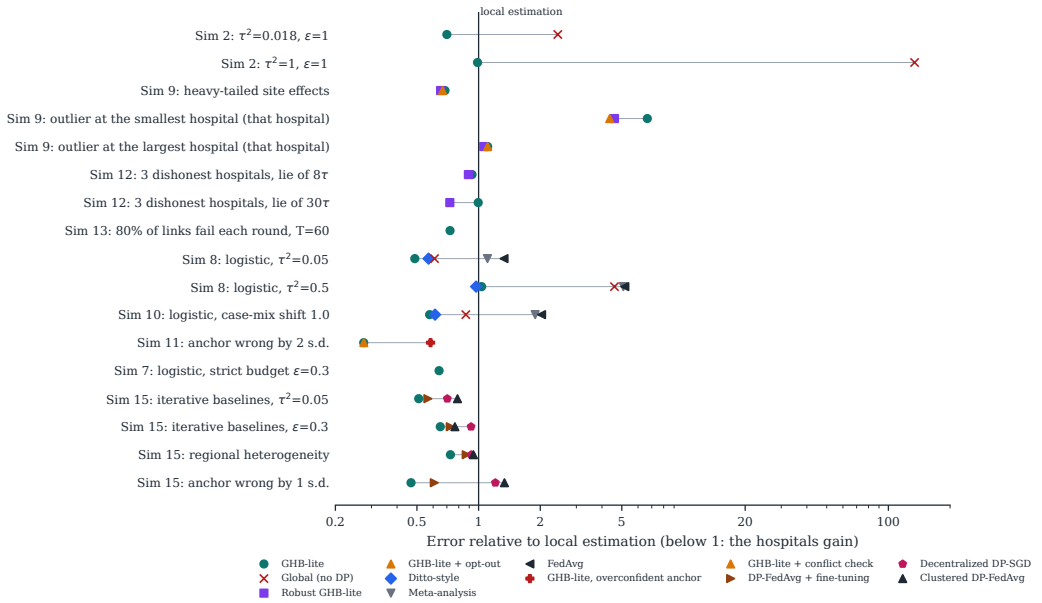


Fig. 12. Summary of the simulation study of the companion paper [Simões et al. 2026]. Each row is a scenario and each marker a method; the horizontal position is the method's error relative to local estimation (mean risk R_k/v_k in the Gaussian simulations, mean excess log-loss divided by that of local estimation in the logistic ones). Left of the vertical line, the hospitals gain. Rows marked "that hospital" report the outlying hospital alone.

that is genuinely unlike the others loses by borrowing, because it cannot tell from its own data that it is atypical.

Real data tell a more demanding story. On networks of a few to a few dozen small hospitals, per-hospital releases at $\epsilon = 1$ carry almost no information, and the lighter design was no better than a hospital that regularizes well alone. The design that the companion paper settles on releases only network sums, and integrates the unknown heterogeneity out against each hospital's own evidence. It was frozen on twelve cancer networks from The Cancer Genome Atlas and confirmed once, under pre-registration, on five networks it had never seen. There it reduced held-out log-loss to 0.928 of local estimation at $\epsilon = 1$ and 0.915 at $\epsilon = 10$. That was lower than every federated and personalized method tuned on the development networks: the best of them reached 0.976 and 0.919. Tuned on the test folds, which no hospital can do, pFedMe was ahead at $\epsilon = 10$ (0.898). Collaboration between small hospitals therefore pays under privacy, but by percent rather than by multiples, and only when the privacy noise is calibrated to the network rather than to each hospital.

Limits. GHB protects patients against honest-but-curious neighbours and any observer of the messages; it does not exclude a hospital that lies, and stronger adversaries call for robust aggregation [Blanchard et al. 2017]. Its privacy guarantee is per release, so refits compose and should be accounted with zero-concentrated or Rényi differential privacy [Bun and Steinke 2016; Mironov 2017]. The Gaussian analysis is asymptotic, and the evidence is simulated: the simulations test internal consistency against adversaries we chose, and say nothing yet about real hospital networks, where effects are not Gaussian and sizes and heterogeneity are not known in advance. Evaluation

on cross-silo benchmarks such as FLamby [Ogier du Terrail et al. 2022] and on real multi-hospital data is the first item of future work. GHB answers the question posed at the start of this section only under these assumptions, and relaxing each of them, together with the measurement problems raised in Sections 2 and 7, defines most of the open problems that follow.

9 OPEN PROBLEMS AND RESEARCH AGENDA

The problems below follow the structure of the paper. The first two concern measuring smallness and evaluating models under it (Sections 2 and 7), the next five arise from the blueprint (Section 8), and the last four concern channels and cells whose potential is not yet matched by evidence (Sections 6 and 7).

(1) **Smallness-aware reporting.** Papers should report n^{eff}, σ and the scarcity profile of Table 2, alongside the TRIPOD+AI items [Collins et al. 2024]. This requires agreed estimators for each factor of Equation (2), especially ρ_S when the deployment population is not yet observed.

(2) **Federated evaluation.** The evaluation of a model is itself small-data (Section 7). Protocols for privacy-preserving external validation across silos, including calibration and subgroup performance, are as important as protocols for training.

(3) **Private estimation of heterogeneity.** The value of collaboration is governed by τ^2 (Equation (5)), yet hospitals rarely know it before joining a consortium. Private, few-round estimators of τ^2 would let a hospital predict its benefit before committing. They would also close the gap that remains in the companion simulation study [Simões et al. 2026]: with the true τ^2, GHB would reach 0.0158 at $\epsilon = 0.3$, against 0.0175 with τ integrated out over its posterior and 0.0269 for local estimation.

(4) **Topology design.** Since mixing depends on the spectral gap (Proposition 8.3), which agreements to sign is an optimization problem with legal and economic constraints. Choosing a few long-range links that maximize the gap is a concrete, under-studied question.

(5) **Small-site privacy.** DP noise scales as n_j^{-2} (Equation (6)). Mechanisms that let small hospitals release jointly, through secure aggregation or shuffling, without a trusted party, would remove the main fairness problem of private pooling.

(6) **Incentives and valuation.** Large hospitals gain little from hierarchical borrowing and contribute most. Data valuation [Ghorbani and Zou 2019] adapted to privatized, decentralized settings could support fair credit, and regulation such as the European Health Data Space [European Parliament and Council of the European Union 2025] will shape what is feasible.

(7) **Shift-aware borrowing.** Proposition 8.1 treats heterogeneity as random. In practice it has structure, through case mix, devices and time. Combining causal invariance [Arjovsky et al. 2019; Castro et al. 2020] with hierarchical borrowing could separate the part of τ^2 that is transportable from the part that is site-specific.

(8) **Priors for clinical tabular data.** TabPFN shows that a well-designed synthetic prior can beat tuned baselines on small tables [Hollmann et al. 2025]. Priors built from clinical knowledge, such as plausible effect directions, monotonicity and interactions, are a natural next step and a principled use of synthetic data that does not conflict with Proposition 4.1, because the information comes from the prior, not from the samples.

(9) **Language models as priors, carefully.** Large language models carry medical knowledge [Singhal et al. 2023] and can act as priors for tabular and few-shot tasks [Hegselmann et al. 2023]. Measuring how much information they actually contribute about a specific local θ, as opposed to population-level generalities, is open.

(10) **Intrinsically small problems.** For conditions where the world supply is below $n^\star$, the right question is how to combine global pooling, mechanistic knowledge and patient-level "n of 1" inference [Estrin 2014; Hekler et al. 2019]. This needs methods that are explicit about which part of the answer comes from which channel.

(11) **The empty cells.** The gap map (Figure 8) lists ten combinations judged promising that the corpus barely touches. Three stand out: methods that down-weight obsolete data in a principled way (T1), which today is handled by ad hoc retraining windows; procurement inside a consortium, where the posterior variance of Section 8 tells every hospital which patients to label next and nobody has studied the joint design (P5 with P1); and pooling aimed at small subgroups rather than small sites (O3 with P1), for which Equation (5) applies with strata in place of hospitals, and which the wider literature has developed for trial design but not for prediction models.

10 CONCLUSION

Small data are the normal condition of clinical machine learning, not an exception to it. Treating smallness as a measurable, task-relative property, through the effective sample size and the smallness index, makes it possible to say how small a problem is and why. The Scarcity-Provenance taxonomy turns that diagnosis into a choice of remedy: identify which term of σ is degraded, then import information from a channel that can address it, keeping in mind that re-expression adds none. Coding 290 works with it shows where the field has looked so far: at label poverty, and at custodial fragmentation through pooling, with very little on obsolete data or noisy measurements, in the corpus and in the wider literature alike, and with multi-institution evidence only for pooling. For the common case of many small hospitals that cannot share data and have no central coordinator, hierarchical borrowing with privatized releases and gossip gives every hospital nearly the same number of virtual patients, pays the privacy cost only in the shared estimate, and runs over whatever collaboration graph exists. On real cancer networks, the gain is a few percent rather than a multiple, and it appears only when the privacy noise is calibrated to the network as a whole; there, the companion paper's method was more accurate than every federated and personalized method tuned as a consortium could tune it. The model in which these results are exact is simple. Simulation in the companion paper shows that its conclusions, that collaboration should be personalized, that privacy should be spent at the population level, and that network topology matters, survive the estimation of every unknown quantity and the move to logistic models, and that with a privatized score release and the heterogeneity integrated out most of the gain survives even a strict privacy budget; whether they hold in real hospital networks is the main question this paper leaves open.

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